

Anatomy label corrections

Leader lines that do not end on the structure they name

Rounds Codex · compiled 16 August 2026

HOW TO READ EVERY FIGURE IN THIS DOCUMENT

- RED** crossed circle — where the leader, dot or badge ends NOW. The number matches the table.
- GREEN** plain circle — where it belongs, with an arrow running from red to green.

Every coordinate is in the SHIPPED page's own pixels, origin top-left. Where a page is off-standard its size is printed in the page header, so the numbers can be scaled if you work from a larger master.

Where a target is given as a coordinate it was measured, and the tissue under it was sampled with the leader's own ink excluded. Where a target is described in words but not numbered, it could not be pinned to a pixel from the shipped render and needs placing from the source file — those are marked PLACE FROM SOURCE and are not guesses to be actioned blindly.

Some rows cannot be fixed by moving anything: the structure the label names is not drawn on the page. Those say so explicitly. They need artwork or a different view, and re-aiming the leader would only move the error.

WHAT IS IN HERE

22 gallery pages, 74 measured findings. These are the pages whose corrections are measured and still open. Pages already corrected and deployed are not included.

PAGE	FINDINGS	WHAT IT IS
Withdrawal — page 2	5	Brain regions affected by withdrawal
Cellulitis — page 2	2	Skin block — subcutaneous layer
Stroke — page 2	2	Carotid & vertebrobasilar systems
Osteoporosis — page 2	4	The skeletal framework — anterior and latera
Aortic dissection — page 2	3	Aortic arch branches
Acute kidney injury — page 2	9	Nephron (functional unit) + blood supply
Bronchiolitis — page 2	5	Airway overview + normal bronchiole cross-se
Appendicitis — page 2	5	Appendix cross-section + relations
BPH — page 2	6	Sagittal male pelvis
Labor — page 2	5	Sagittal pelvis at term
Sepsis — page 2	2	Organ callouts
DVT — page 2	1	Deep venous anatomy of the leg
Spinal cord injury — page 2	1	Major tracts (cord cross-section)
Compartment syndrome — page 2	2	Leg compartments (cross-section)

Pulmonary embolism — page 2	8	Central heart figure + vascular landmarks
Anxiety — page 2 [NEW, 2026-08-16]	3	Key brain structures — circuit figure
Bronchiolitis — page 7 [NEW, 2026-08-16]	1	Airway cross-section, bronchiolitis side
ACS — page 2	1	Coronary anatomy (numbered badges)
ACS — page 6	1	Magnifier ring on the anterior heart
Aortic stenosis — page 2	1	Cut heart
Cardiomyopathy — page 1	4	One heart split into three phenotype slabs
Pericarditis — page 4	3	ECG strip

How to make the corrections

1. Moving a leader tip or a dot — most of this document

Erase the last segment along the LEADER'S OWN AXIS, then redraw to the green point. Two things keep it invisible: take the stroke width and colour by sampling the leader itself a little way back from the tip, and erase along the axis rather than with a box, so the surrounding artwork is never touched.

The test that the move is clean: at the OLD endpoint, ink in a 13x13 box should fall to ZERO. If ink remains there, the leader was extended rather than moved, and the page now carries two terminators.

2. Never repaint artwork

If a fix would mean painting over illustration - erasing a disc of shaded tissue, rebuilding a background, redrawing a structure - stop and mark the page for a re-render instead. This has been attempted here and it does not repair cleanly; every large tissue-side erase left a visible patch. Several rows in this document say exactly that and are addressed to the illustrator, not to Photoshop.

3. Rows that no move can fix

Where the structure the label names is NOT DRAWN on the page, re-aiming the leader only moves the error somewhere else. Those rows say so in the text and need artwork or a different view - for example a lateral inset, an added slab, or a redrawn tracing. Rows marked PLACE FROM SOURCE are ones we could not pin to a pixel from the shipped render; place them from the layered file.

4. Save at the SAME pixel dimensions

Do not resample, resize, crop or rotate. The coordinates in this document are in the shipped page's own pixels, and the app, the thumbnails and the download PDF are all built from that geometry. Save as JPEG at high quality - we re-encode on integration at the file's own quantisation tables and chroma sampling, so an exact quality match on your side is not needed, but a resize breaks everything downstream.

5. Send the corrected page back; do not rebuild anything else

We regenerate the thumbnail and rebuild that gallery's download PDF. That second step matters: the download PDF is a separate build artefact and drifts from the pages if it is not rebuilt - four galleries shipped for months with an old logo in the PDF while the app showed the current one.

6. One page at a time is fine

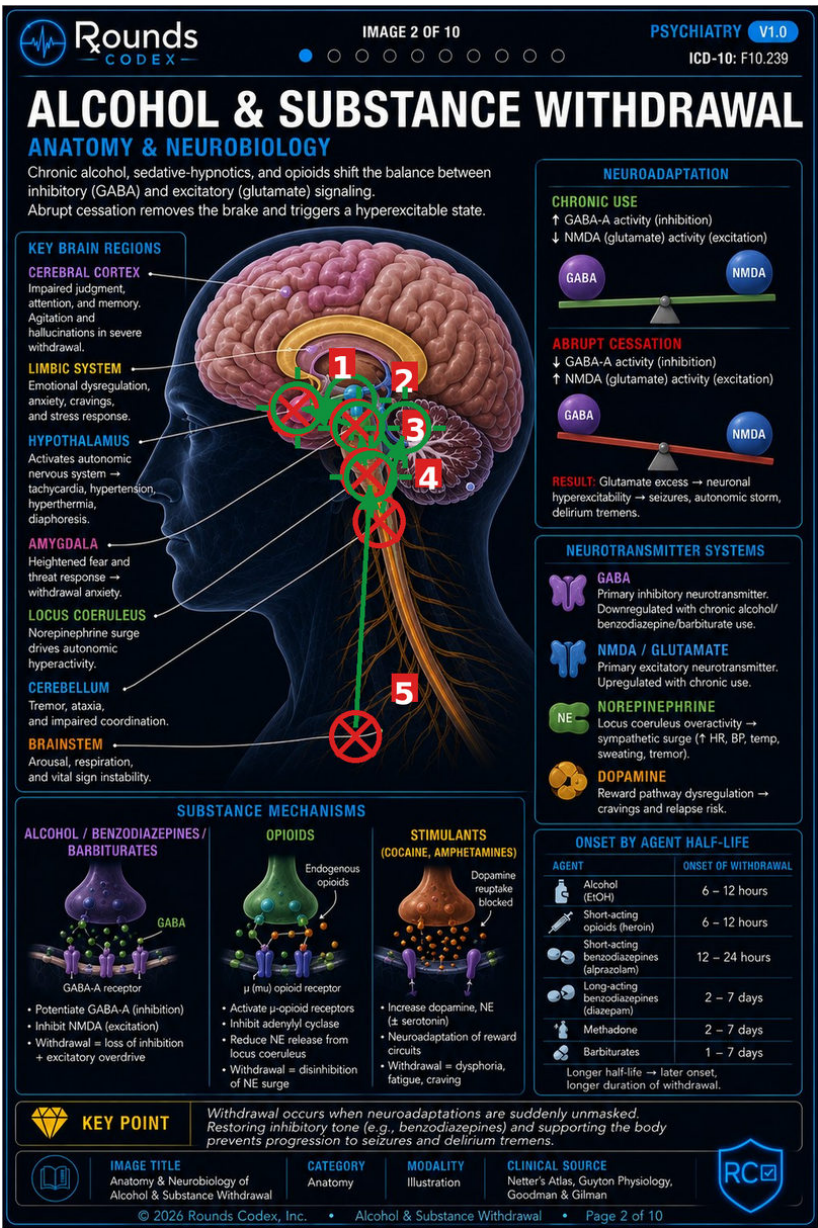
Corrections are integrated and deployed in batches of about six pages. Each one is measured against the sheet before it ships - what was fixed, what was missed, and what was left - and that measurement is reported back honestly rather than assumed.

Withdrawal — page 2

Brain regions affected by withdrawal · withdrawal-02.jpg · 1024×1536 px · 5 findings

All 5 rows measured and confirmed. Local fix: 5 dot moves.

The cleanest instance of the displaced-series fault in the whole review, and confirmable end to end by colour with no anatomical judgement: each label takes the NEXT label's structure, and the last falls off the figure entirely. The page prints its own colour key, so every row below is settled by sampling the blob under the dot and looking the colour up in that key.



WHAT TO CHANGE

- 1 **HYPOTHALAMUS**
NOW (365,509) the PINK blob — Amygdala's colour
MOVE TO (437,495) the BLUE blob, sampled (14,96,148) against the key's (34,135,193)
- 2 **AMYGDALA**
NOW (443,527) the GREEN blob — Locus coeruleus's colour
MOVE TO (370,505) the PINK blob, sampled (185,75,118) against the key's (197,102,161)
- 3 **LOCUS COERULEUS**
NOW (457,588) the ORANGE brainstem, 28 px below the green stalk's end
MOVE TO (447,530) the GREEN blob, sampled (75,123,46) against the key's (139,187,111)
- 4 **CEREBELLUM**
NOW (473,648) the ORANGE cord, 60 px lower again
MOVE TO (505,530) the mauve arbor-vitae mass, sampled (85,52,62)
- 5 **BRAINSTEM**
NOW (443,918) EMPTY DARK BACKGROUND, sampled (6,13,22) — the cord at that height is 170–200 px to the right
MOVE TO (462,592) the orange brainstem/cord, sampled (161,94,56)

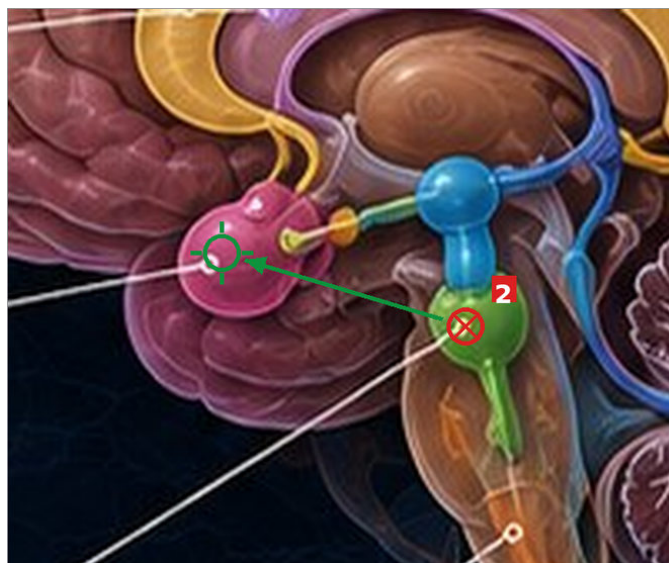
Withdrawal — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



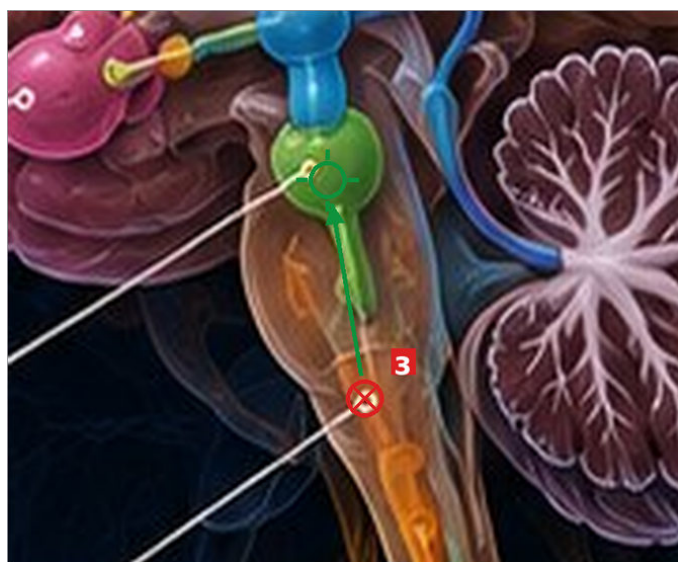
1 HYPOTHALAMUS

(365,509) the PINK blob — Amygdala's colour
-> (437,495) the BLUE blob, sampled (14,96,148) against
the key's (34,135,193)



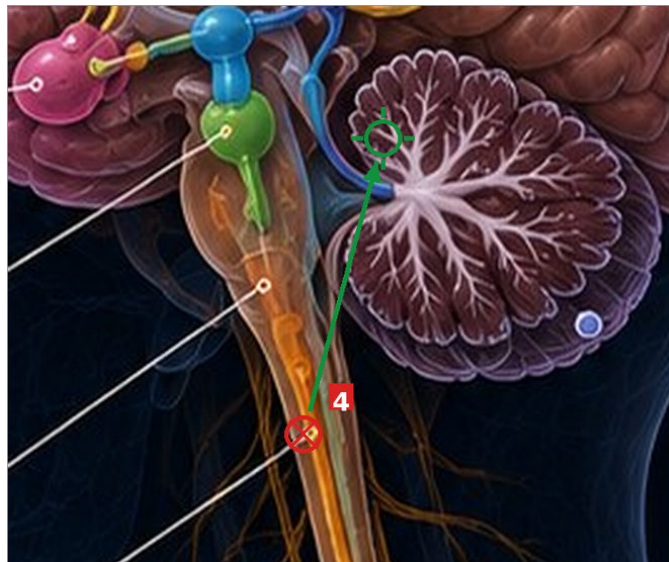
2 AMYGDALA

(443,527) the GREEN blob — Locus coeruleus's colour
-> (370,505) the PINK blob, sampled (185,75,118) against
the key's (197,102,161)



3 LOCUS COERULEUS

(457,588) the ORANGE brainstem, 28 px below the green
stalk's end
-> (447,530) the GREEN blob, sampled (75,123,46) against
the key's (139,187,111)

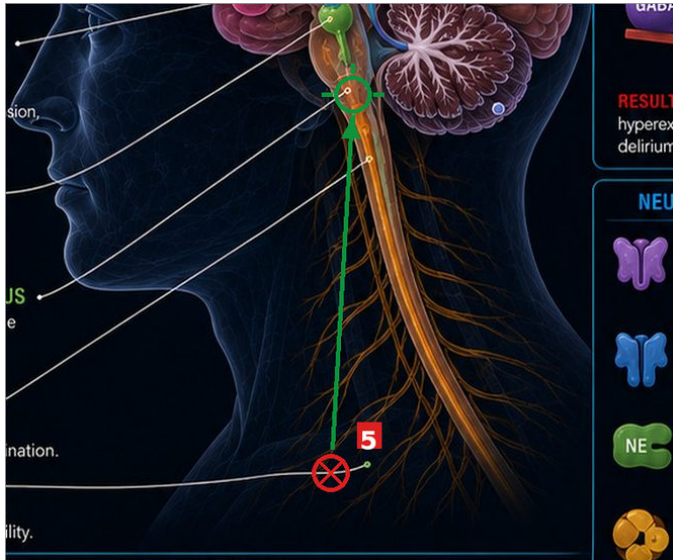


4 CEREBELLUM

(473,648) the ORANGE cord, 60 px lower again
-> (505,530) the mauve arbor-vitae mass, sampled
(85,52,62)

Withdrawal — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



5 BRAINSTEM

(443,918) EMPTY DARK BACKGROUND, sampled (6,13,22) — the
cord at that height is 170–200 px to the right
-> (462,592) the orange brainstem/cord, sampled
(161,94,56)

Skin block — subcutaneous layer · cellulitis-02.jpg · 1024×1536 px · 2 findings

2 of 4 rows measured and confirmed; 2 carried.

The vessels are drawn correctly and in the conventional colours; the two labels have moved one step along. Settled by sampling, not by judgement — the same shape as hepatitis p2 and ckd p2.

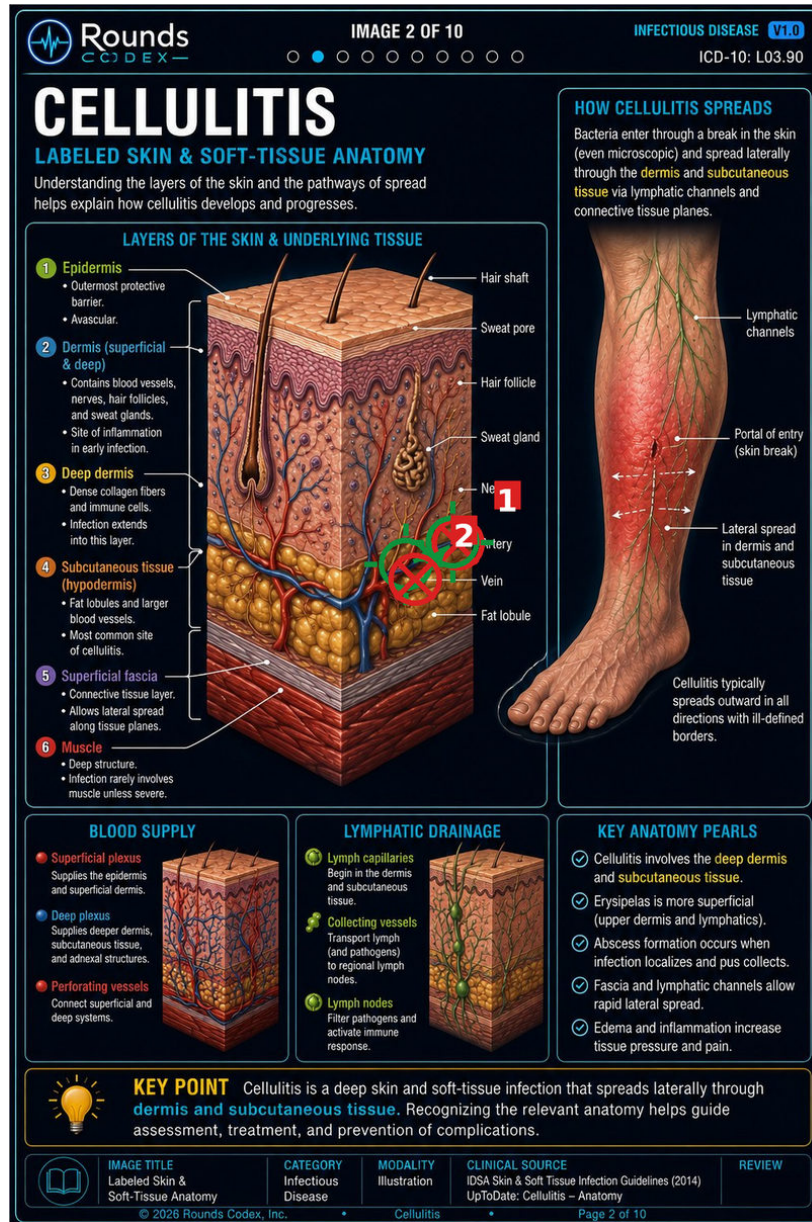
WHAT TO CHANGE

1 Artery

NOW (563,677) the BLUE vessel, sampled (56,66,78) — matches the vein at (556,675)
MOVE TO (497,703) the RED vessel, sampled (125,32,25). Drawn, unambiguous, and currently unlabelled

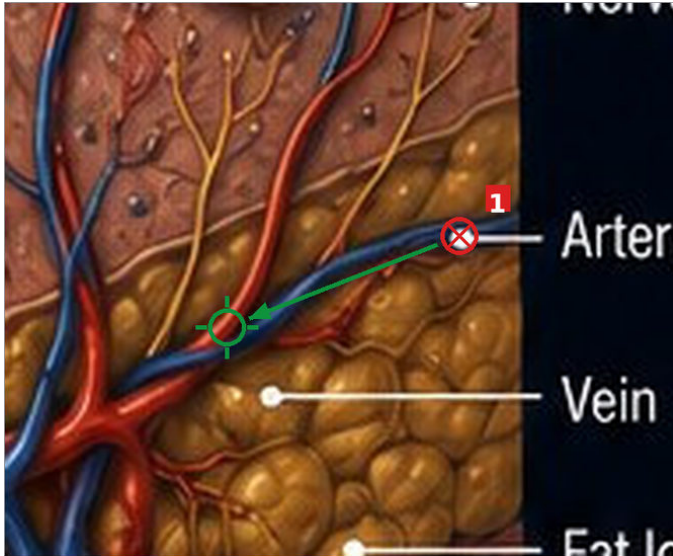
2 Vein

NOW (509,722) a GOLD FAT LOBULE, sampled (143,99,62). No vessel within 12 px
MOVE TO (556,675) the blue vessel — the point the Artery leader currently occupies



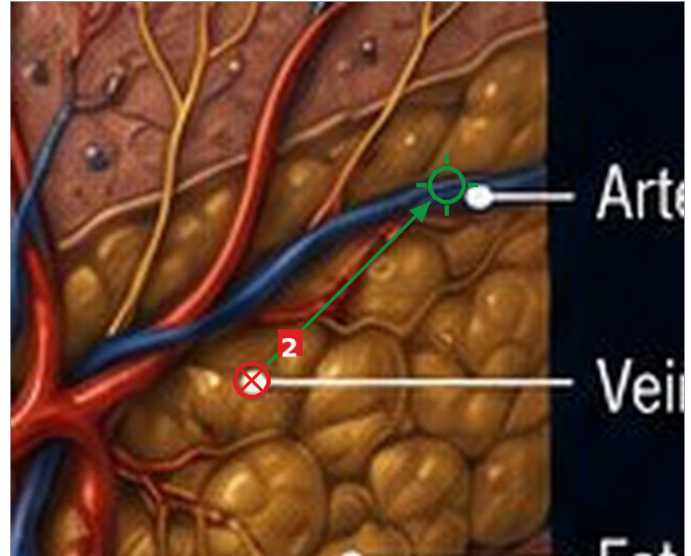
Cellulitis — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 Artery

(563,677) the BLUE vessel, sampled (56,66,78) — matches the vein at (556,675)
-> (497,703) the RED vessel, sampled (125,32,25). Drawn, unambiguous, and currently unlabelled



2 Vein

(509,722) a GOLD FAT LOBULE, sampled (143,99,62). No vessel within 12 px
-> (556,675) the blue vessel — the point the Artery leader currently occupies

CARRIED FROM THE WORK ORDER - NOT RE-MEASURED HERE

3 Hair follicle

plain reticular dermis on the block's RIGHT SIDE FACE; the sheathed follicle is drawn on the FRONT face at left. Check the whole right-hand column against which face each structure is drawn on — Sweat gland, Nerve and Hair shaft sit in the same column

4 Portal of entry (skin break) [CHECK]

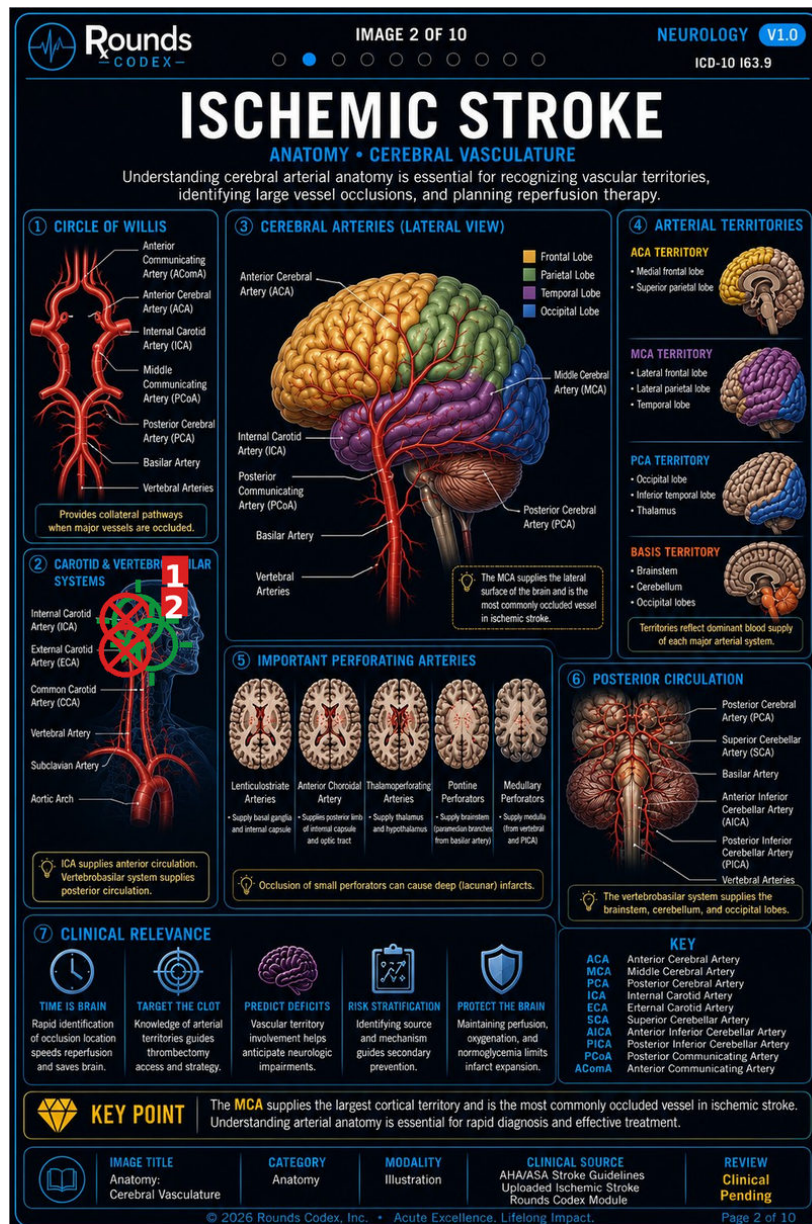
erythematous skin ~30 px right of the wound. Move to the dark slit-shaped break itself

Stroke — page 2

Carotid & vertebrobasilar systems · stroke-02.jpg · 1024×1536 px · 2 findings

9 rows flagged; the two carotid rows measured at 11×.

The panel draws two red trunks ascending the neck with the face to the right. The anterior trunk (x 160–168) is the common carotid and bifurcates at (163,800): one limb continues to the skull base giving no facial branches (the ICA), the other fans into the face (the ECA). Both carotid labels have crossed to the posterior trunk — the vertebral artery.



WHAT TO CHANGE

- 1 Internal Carotid Artery (ICA)**
NOW (146,768) the VERTEBRAL artery
MOVE TO (161,770) the post-bifurcation limb with no facial branches
- 2 External Carotid Artery (ECA)**
NOW (145,808) the VERTEBRAL artery; the leader's final dash reaches x 158–160, the COMMON carotid below the bifurcation
MOVE TO (178,800) the anterior branch fanning into the face

Stroke — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 Internal Carotid Artery (ICA)

(146,768) the VERTEBRAL artery

-> (161,770) the post-bifurcation limb with no facial branches



2 External Carotid Artery (ECA)

(145,808) the VERTEBRAL artery; the leader's final dash reaches x 158–160, the COMMON carotid below the bifurcation

-> (178,800) the anterior branch fanning into the face

Osteoporosis — page 2

The skeletal framework — anterior and lateral skeletons · osteoporosis-02.jpg · 1024×1536 px · 4 findings

2 of 5 rows measured and confirmed; 3 carried.

Tibia and Fibula are on each other's bone. The tibia is the BROAD weight-bearing shaft and the fibula the SLENDER one; both labels point at the slender bone on the anterior figure, and on the lateral figure both fall on the same pair the other way round.


Rounds
 — CODEX —

IMAGE 2 OF 10

MSK & RHEUM V1.0

ICD-10: M81.0

OSTEOPOROSIS

ANATOMY: THE SKELETAL FRAMEWORK

Bones are dynamic living tissue composed of cortical and trabecular bone. Healthy microarchitecture provides strength; loss of this architecture leads to fragility and fractures.

KEY ANATOMY

Vertebrae

Rich in trabecular bone. Common site of compression fractures.

Proximal Femur

Contains both cortical (outer) and trabecular bone. Common site of hip fractures.

Distal Radius

Predominantly trabecular bone. Common site of Colles fractures.

Ribs & Pelvis

Provide structural support and protection; composed of cortical shell with trabecular interior.

BONE MICROARCHITECTURE

NORMAL BONE

- Thick, well-connected trabeculae create a strong, dense network.
- More resistant to stress.

OSTEOPOROTIC BONE

- Thinned, sparse trabeculae with loss of connections.
- Weakened structure increases fragility and fracture risk.

BONE COMPOSITION

- CORTICAL BONE**
 - Dense, compact outer layer.
 - Provides strength and resists bending.
- TRABECULAR BONE**
 - Spongy, lattice-like inner structure.
 - Distributes load and absorbs stress.

BONE IS A DYNAMIC TISSUE

Continuous remodeling maintains bone strength.

Balanced remodeling = strong bone

TYPICAL VERTEBRA (CROSS-SECTION)

- Cortical bone**
 - Dense outer shell resists compression.
- Trabecular bone**
 - Internal lattice distributes load and absorbs stress.

COMMON SKELETAL SITES AT RISK

Spine

Vertebral compression fractures.

Hip

Femoral neck and intertrochanteric fractures.

Wrist

Distal radius (Colles) fractures.

ANATOMY KEY TAKEAWAYS

- Bones are living, dynamic tissue made of cortical and trabecular bone.
- Trabecular bone is metabolically active and more vulnerable to loss in osteoporosis.
- Vertebrae, hip, and distal radius are the most common fracture sites.
- Loss of bone microarchitecture and connectivity leads to weakened bones and increased fracture risk.

KEY POINT

Strong bones depend on good architecture and balance. Osteoporosis disrupts this balance, weakening bones silently until a fracture reveals the disease.

IMAGE TITLE

Anatomy: The Skeletal Framework

CATEGORY

Anatomy

MODALITY

Illustration

CLINICAL SOURCE

Drake RL, Vogl W, Mitchell AWM. Gray's Anatomy for Students. 5th Ed. Moore KL. Clinically Oriented Anatomy. 8th Ed.

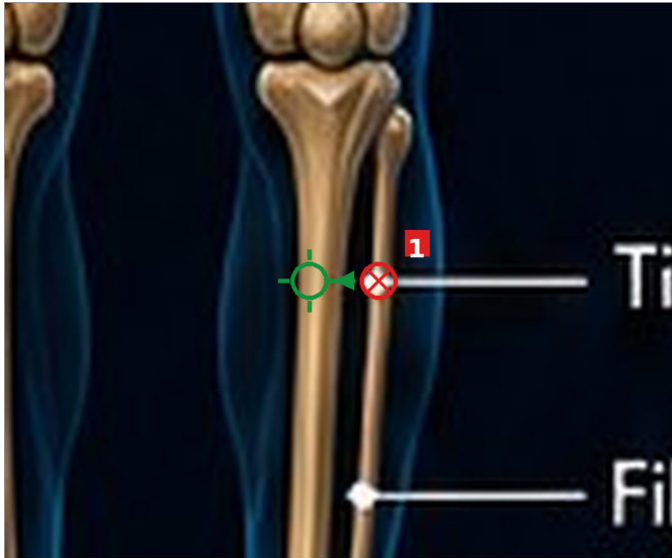
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WHAT TO CHANGE

- 1 Tibia (anterior skeleton)**
NOW (449,691) the SLENDER shaft — at that height it spans x 448–450
MOVE TO (436,691) the broad shaft
- 2 Fibula (anterior skeleton)**
NOW (446,730) the SLENDER shaft again, on its medial edge
MOVE TO (449,730) the slender shaft proper — but note both current tips are on ONE bone
- 3 Tibia (lateral skeleton)**
NOW (558,673) the SLENDER anterior shaft (x 552–560) at the knee
MOVE TO the broad shaft
- 4 Fibula (lateral skeleton)**
NOW (563,708) the BROAD shaft behind it (x 561–572)
MOVE TO the slender shaft

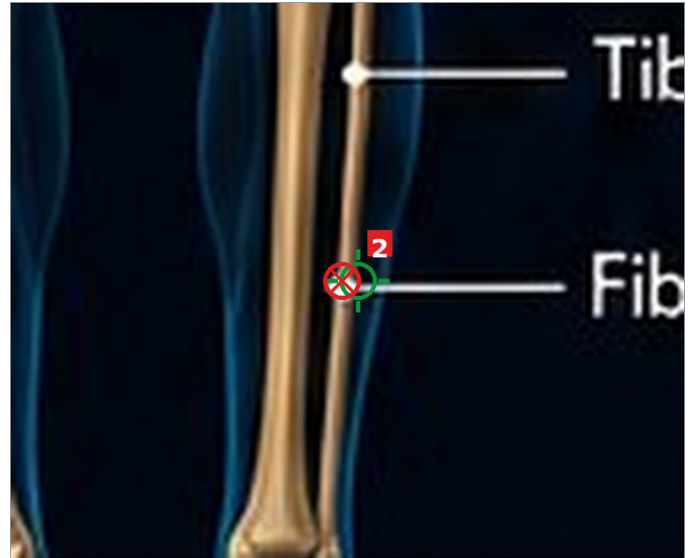
Osteoporosis — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 Tibia (anterior skeleton)

(449,691) the SLENDER shaft — at that height it spans x 448–450
-> (436,691) the broad shaft



2 Fibula (anterior skeleton)

(446,730) the SLENDER shaft again, on its medial edge
-> (449,730) the slender shaft proper — but note both current tips are on ONE bone



3 Tibia (lateral skeleton)

(558,673) the SLENDER anterior shaft (x 552–560) at the knee
-> the broad shaft



4 Fibula (lateral skeleton)

(563,708) the BROAD shaft behind it (x 561–572)
-> the slender shaft

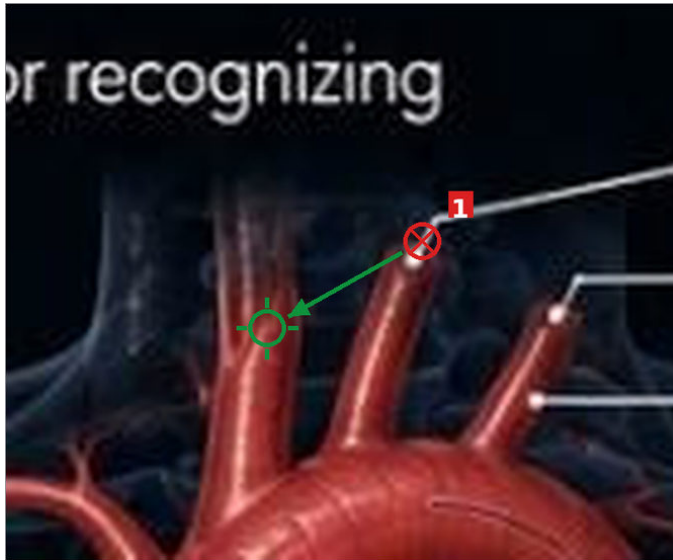
CARRIED FROM THE WORK ORDER - NOT RE-MEASURED HERE

— Ribs / Humerus / Radius

reported by the order as Ribs → the humerus, Humerus → the vertebral column, Radius → ... — not re-measured here

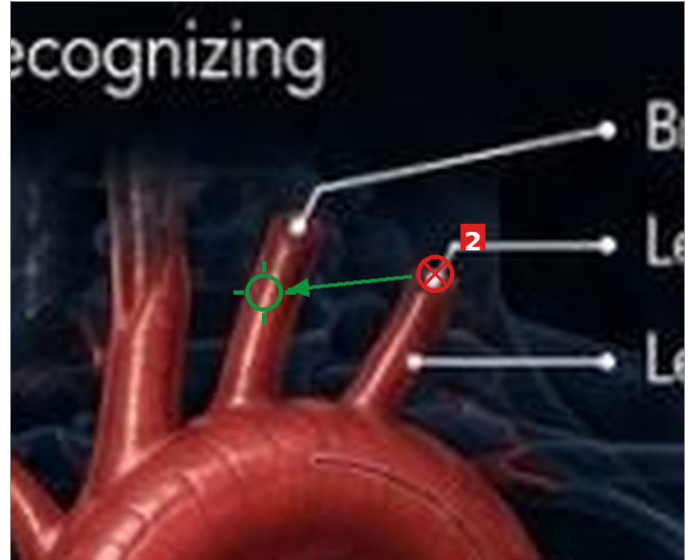
Aortic dissection — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



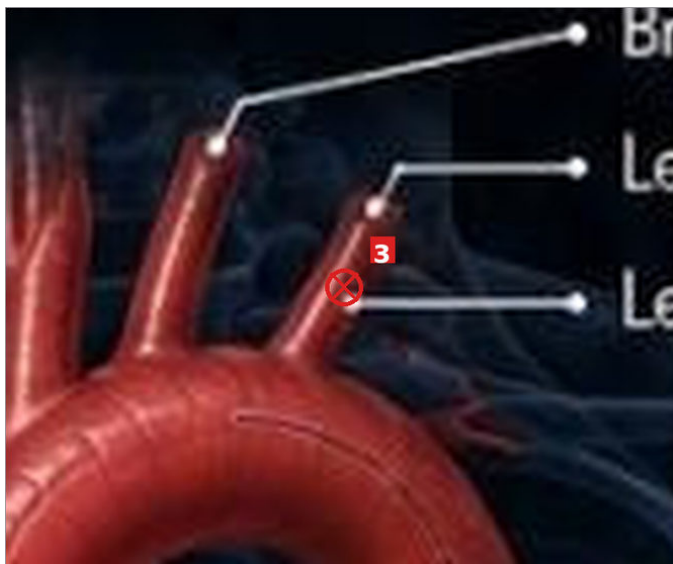
1 Brachiocephalic trunk

(462,222) branch 2 — the left common carotid
-> (430,240) branch 1, the thick bifurcating trunk



2 Left common carotid artery

(490,236) branch 3 — the left subclavian
-> (453,240) branch 2



3 Left subclavian artery

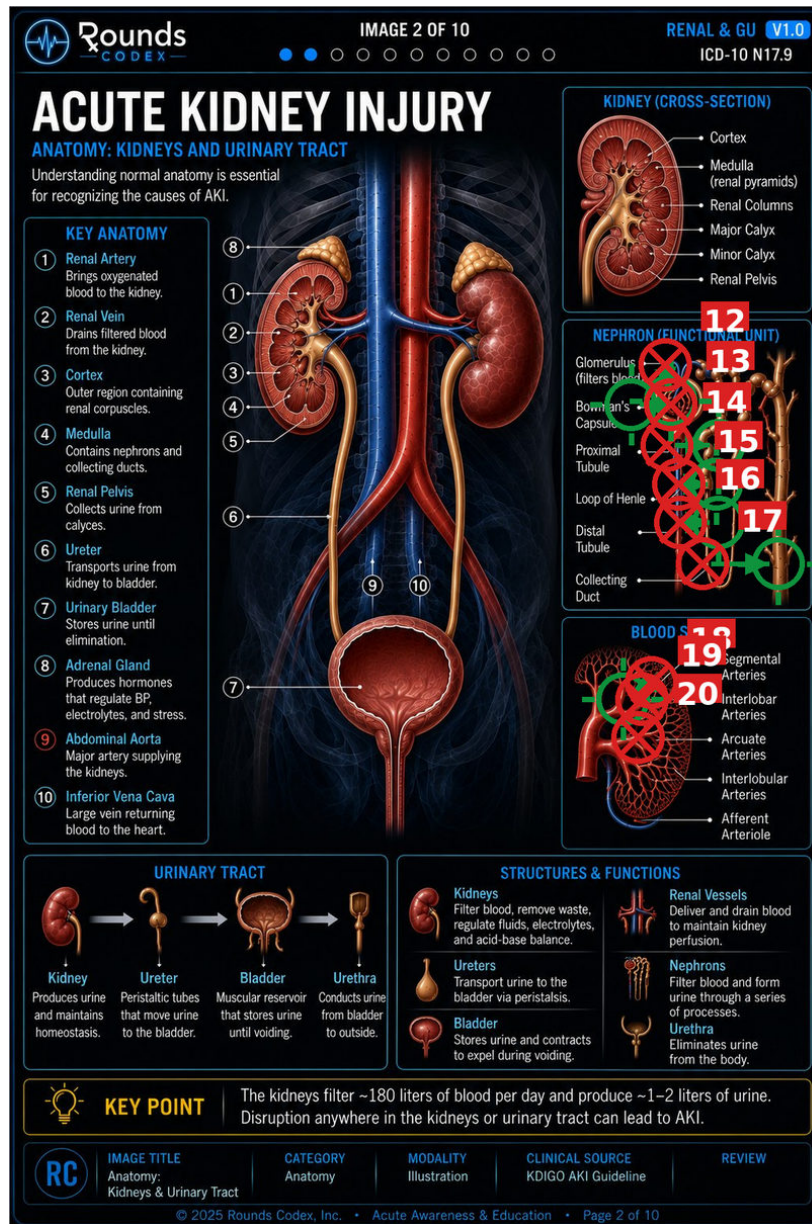
(484,252) branch 3 — CORRECT, but it currently shares
that branch with the row above
-> stays where it is

Acute kidney injury — page 2

Nephron (functional unit) + blood supply · aki-02.jpg · 1024×1536 px · 9 findings

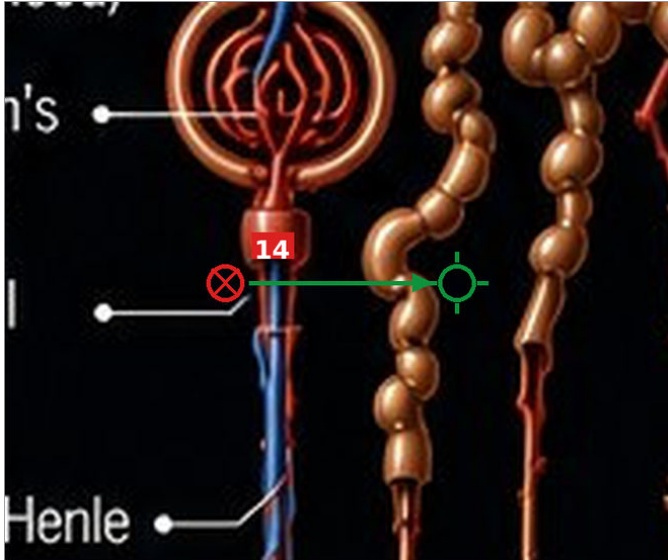
The largest page in the order: 27 labels, 15 to move, 4 to check. Nephron panel verified completely.

The nephron panel draws, left to right: the vasa recta bundle as a blue/red vertical pair at x 833–847, then the gold tubules — beaded proximal convolutions at x 855–925, hairpin limbs at x 865–885 and 895–910, and the thick collecting-duct trunk at x 950–980. Four leaders leave a stacked label column and stop at the first thing they meet at their own height, which is the blood vessel.



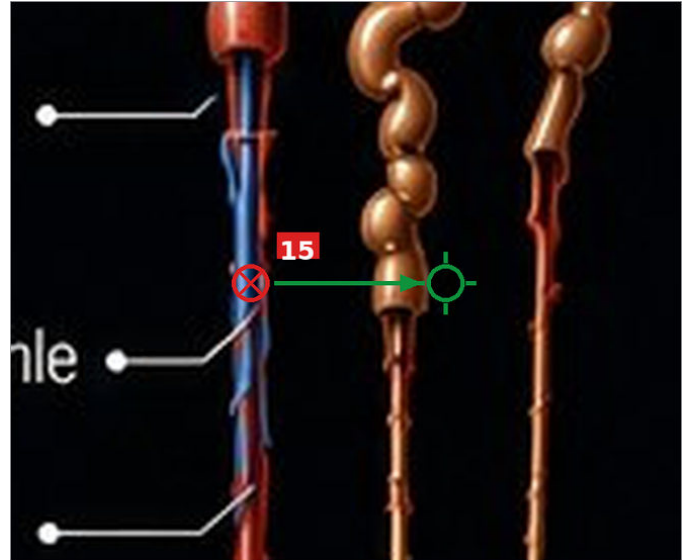
Acute kidney injury — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



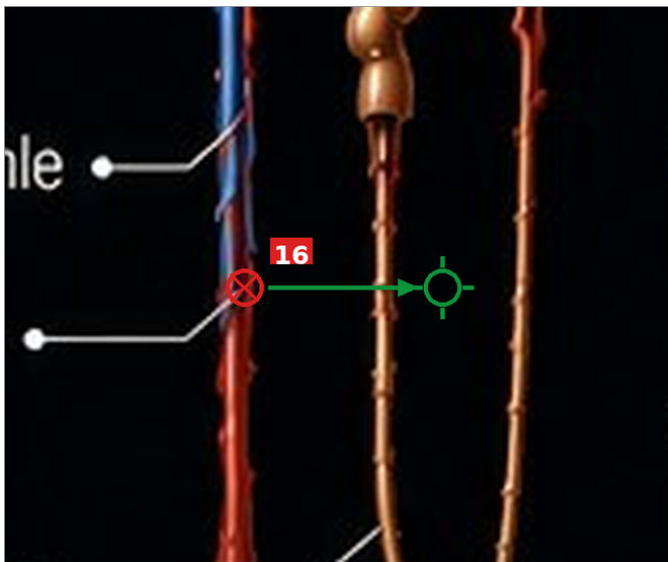
14 Proximal Tubule

(825,551) the vasa recta bundle (a 17-px-wide blood vessel)
-> (890,551) the beaded gold tubule, x 860–925



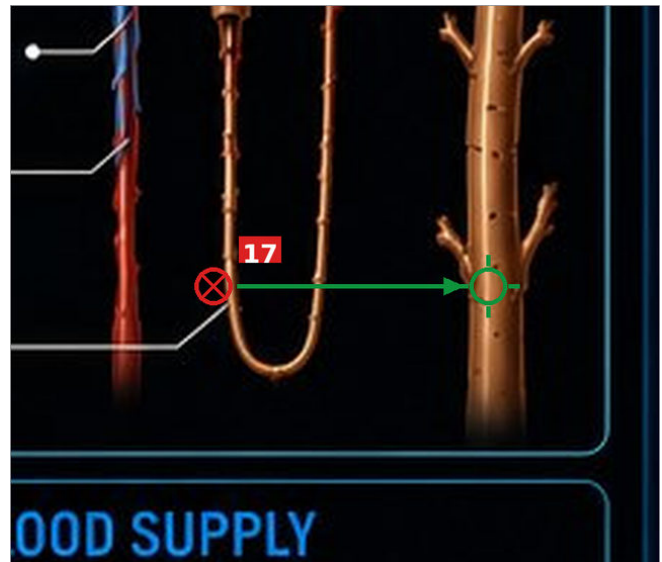
15 Loop of Henle

(840,600) the vasa recta bundle
-> (887,600) the gold hairpin, x 865–910



16 Distal Tubule

(842,648) the red vessel of the same bundle
-> (890,648) the gold tubule returning to the glomerulus

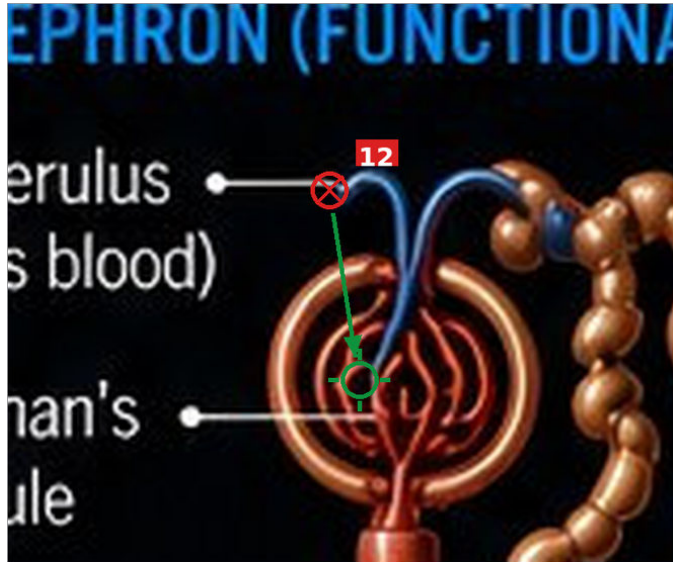


17 Collecting Duct

(870,700) the descending gold limb of Henle — one structure short
-> (965,700) the thick gold trunk, x 950–980

Acute kidney injury — page 2 — close-ups

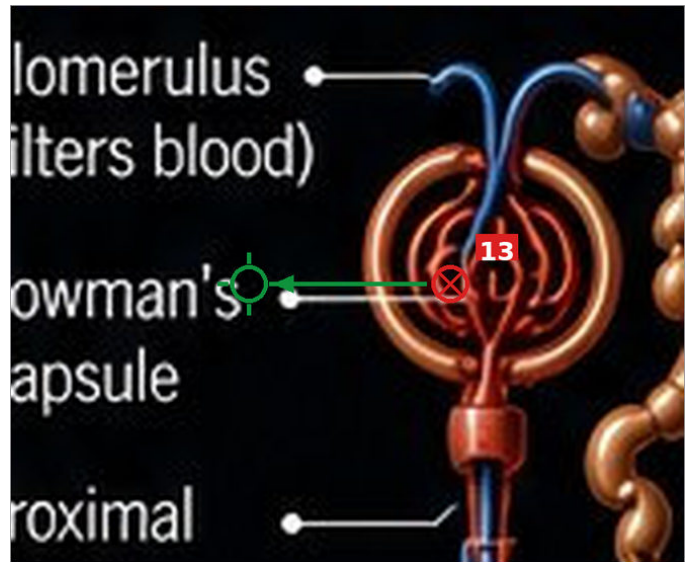
red crossed circle = where it ends now green plain circle = where it belongs



12 Glomerulus (filters blood)

(823,450) the bend of the blue arteriole, above the capsule

-> (830,495) the red glomerular tuft



13 Bowman's Capsule

(830,500) the red glomerular tuft, inside the capsule

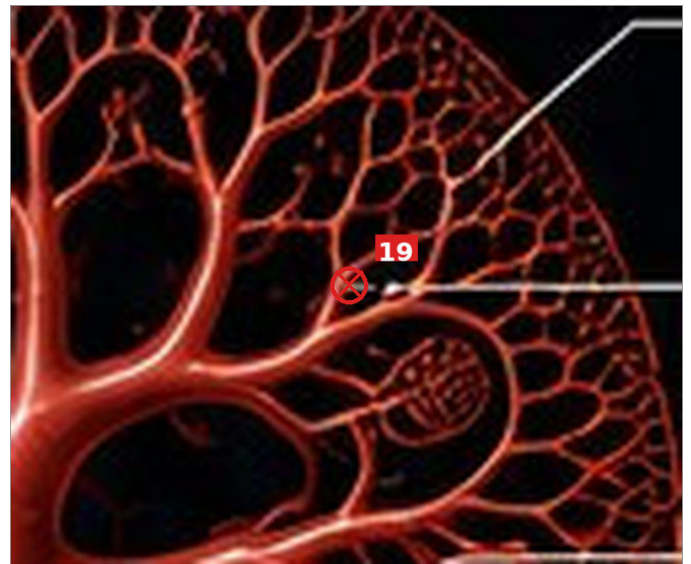
-> (780,500) the peach ring (x 775–850, y 460–540) — its left limb



18 Segmental Arteries

(806,849) out among the fine peripheral cortical branches

-> (770,870) the thick first-order trunks, x 750–790, y 855–890



19 Interlobar Arteries

(794,870) 12 px MORE MEDIAL than Segmental — the series runs backwards

-> lateral of the corrected Segmental endpoint

Acute kidney injury — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



20 Arcuate Arteries

(789,918) more medial still

-> lateral again, at the corticomedullary arcs

NOTES

Anatomically these run medial→lateral as segmental → interlobar → arcuate → interlobular. Measured at 11× they are 806, 794, 789, 838 — the first three exactly backwards, with Interlobular alone where it belongs. NOTE: an earlier draft of this sheet told you the inversion could not be reproduced. That draft was wrong and was withdrawn; it had been read at 5×, where the difference is smaller than the stroke widths.

Bronchiolitis — page 2

Airway overview + normal bronchiole cross-section · bronchiolitis-02.jpg · 1024×1536 px · 5 findings

9 rows flagged.

A side inversion found by a laterality sweep, not by the page-by-page review. The trachea descends at x 470–490 and the carina is at (472,625). On a front-facing figure, viewer's left = patient's RIGHT. Both bronchus labels are in one lung, and it is the wrong one for the first — and underneath that, neither leader reaches a bronchus at all: both stop in parenchyma 50–90 px short of the ringed tubes.

IMAGE 2 OF 10

OB & PEDS V1.0

ICD-10: J21.0

BRONCHIOLITIS (RSV)

ANATOMY: INFANT LOWER AIRWAY

Bronchiolitis affects the small airways (bronchioles) of infants, causing inflammation, mucus accumulation, and air trapping that lead to wheezing, crackles, and impaired gas exchange.

CROSS-SECTION: NORMAL BRONCHIOLE

Normal bronchioles are open and lined with ciliated epithelium that moves mucus out of the airway.

CROSS-SECTION: INFECTED BRONCHIOLE (BRONCHIOLITIS)

Inflammation, mucus, and cellular debris narrow the airway, causing obstruction and impaired airflow.

ALVEOLI: SITE OF GAS EXCHANGE

Oxygen passes into the blood and carbon dioxide is removed at the alveoli.

AIRFLOW EFFECT IN BRONCHIOLITIS

NORMAL AIRFLOW

Air moves freely in and out.

AIR TRAPPING

Obstruction creates a ball-valve effect, trapping air and causing hyperinflation.

KEY ANATOMY PEARLS

- Infants have smaller airways (narrow diameter) that are easily obstructed.
- Ciliated epithelium clears mucus – infection damages cilia and impairs clearance.
- Bronchiolitis involves the bronchioles, not the larger bronchi or alveoli.
- Inflammation + mucus = narrowed airways, air trapping, and hypoxia.

KEY POINT

Bronchiolitis is a disease of the bronchioles in infants – inflammation, mucus, and sloughed cells narrow the small airways and trap air.

IMAGE TITLE	CATEGORY	MODALITY	CLINICAL SOURCE
Anatomy: Infant Lower Airway	Anatomy	Illustration	AAP Clinical Practice Guideline (2014, Updated 2019), Red Book 2024

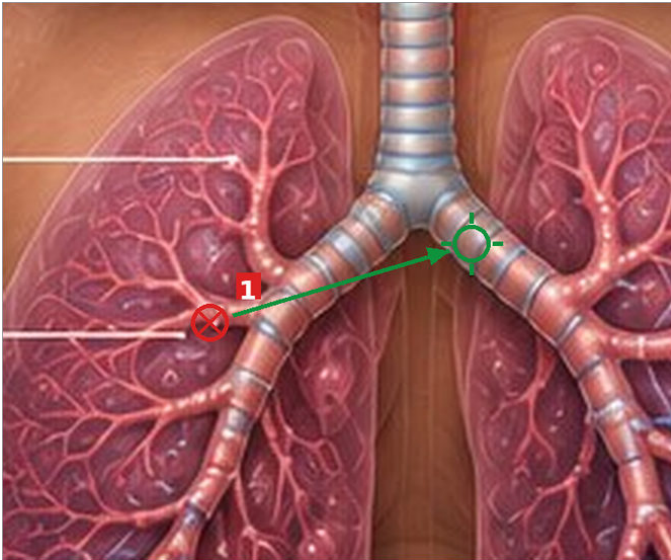
© 2026 Rounds Codex, Inc. · Bronchiolitis (RSV) · Page 2 of 10

WHAT TO CHANGE

- Left main bronchus**
 NOW (408,660) viewer's-left lung = the patient's RIGHT lung, and in parenchyma, not on a tube
 MOVE TO (494,634) the left main bronchus running down-right from the carina
- Right main bronchus [CHECK]**
 NOW (416,606) a small upper-lobe segmental branch, in the same lung as row 1
 MOVE TO (444,634) the right main bronchus
- Pharynx**
 NOW (389,450) inside the ORAL CAVITY, just behind the lips
 MOVE TO (460,455) the vertical pharyngeal column, 70 px posterior
- Oral cavity**
 NOW (382,427) the HARD PALATE / nasal floor
 MOVE TO (385,450) the oral cavity below the palate
- Nasal cavity [CHECK]**
 NOW (384,385) the SKIN of the nasal dorsum
 MOVE TO (384,397) inside the turbinate-containing cavity, ~12 px lower

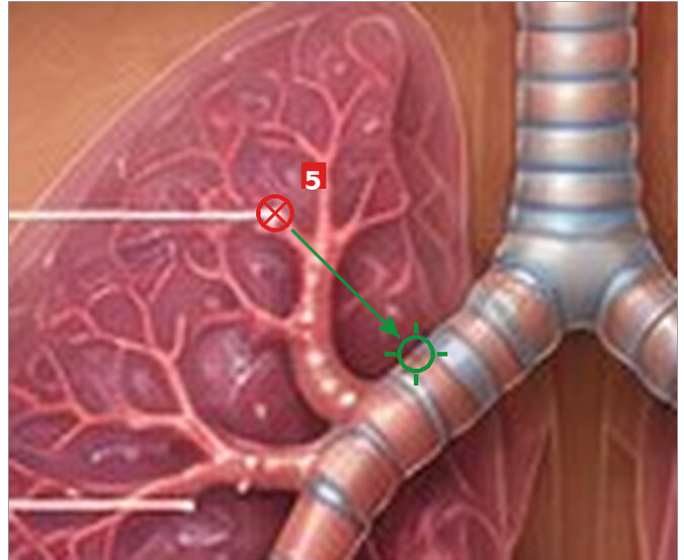
Bronchiolitis — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



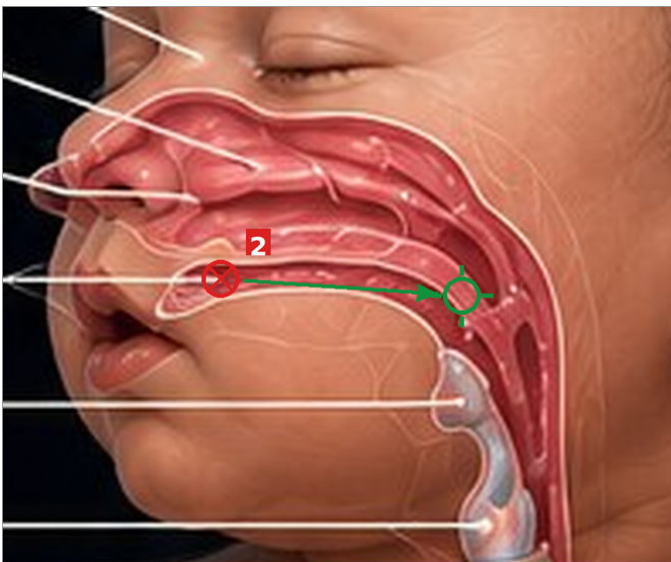
1 Left main bronchus

(408,660) viewer's-left lung = the patient's RIGHT lung, and in parenchyma, not on a tube
 -> (494,634) the left main bronchus running down-right from the carina



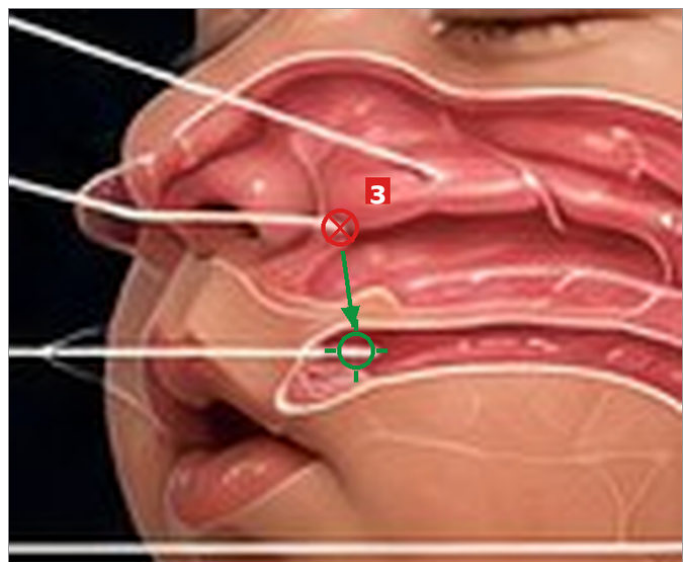
5 Right main bronchus [CHECK]

(416,606) a small upper-lobe segmental branch, in the same lung as row 1
 -> (444,634) the right main bronchus



2 Pharynx

(389,450) inside the ORAL CAVITY, just behind the lips
 -> (460,455) the vertical pharyngeal column, 70 px posterior

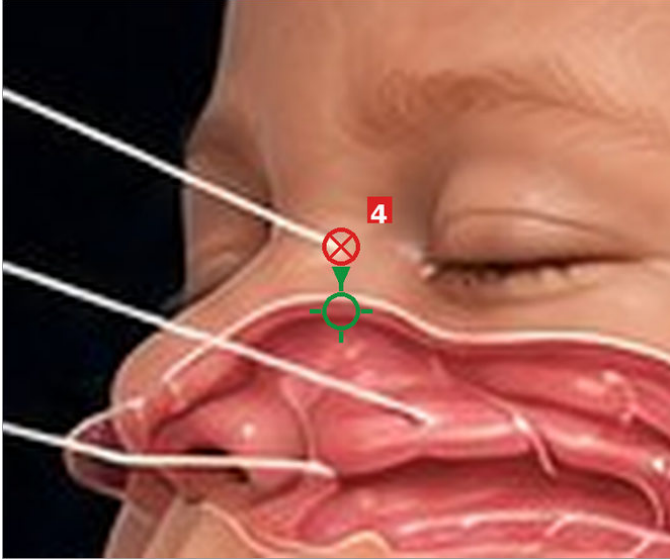


3 Oral cavity

(382,427) the HARD PALATE / nasal floor
 -> (385,450) the oral cavity below the palate

Bronchiolitis — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



4 Nasal cavity [CHECK]

(384,385) the SKIN of the nasal dorsum

-> (384,397) inside the turbinate-containing cavity, ~12
px lower

NOTES

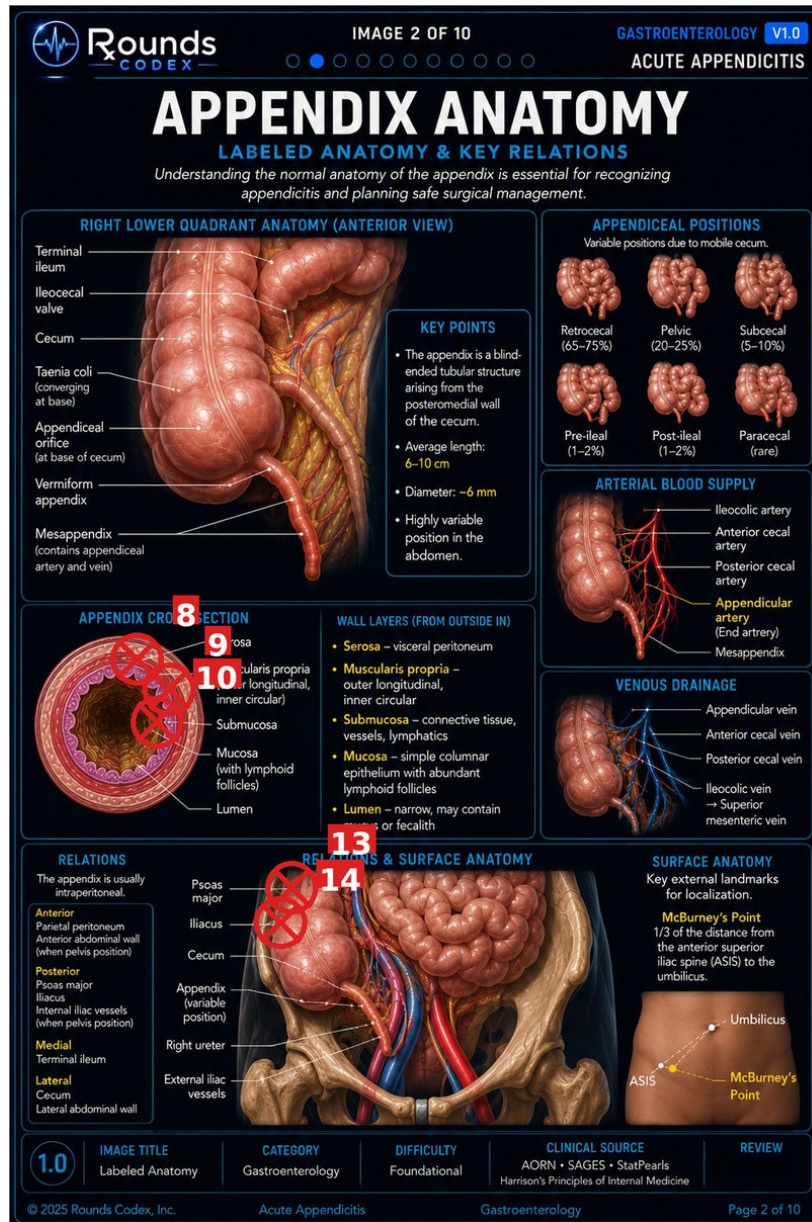
Rows 2–4 are a displaced series: pharynx lands where oral cavity belongs, oral cavity where nasal cavity belongs, nasal cavity on skin above the nose. Separately, the NORMAL BRONCHIOLE cross-section has its three layer labels in REVERSE RADIAL ORDER — Smooth muscle on the gold mucus at radius ~49 when it belongs at ~66, and Mucus layer on the epithelial band. Please check those two radii at source; we could not measure them numerically.

Appendicitis — page 2

Appendix cross-section + relations · appendicitis-02.jpg · 1024×1536 px · 5 findings

6 of 11 measured and confirmed; 5 carried.

The cross-section is concentric, so radius settles it. Fitted from its own outline: CENTRE (134,900), RADIUS 110. Reading outward the layers are lumen r 0–60, mucosa r 61–75, submucosa r 76–90, muscularis propria r 91–108, serosa r 109–110. Four of the five layer labels sit one to two layers too deep and the fifth too shallow.



WHAT TO CHANGE

8 Serosa

NOW (160,821) r = 83 — the dark red
SUBMUCOSA
MOVE TO the outer rim, r ≈ 110

9 Muscularis propria

NOW (200,859) r = 78 — the
mucosa/submucosa boundary
MOVE TO the pale speckled band, r 91–108

10 Submucosa

NOW (185,900) r = 51 — the orange LUMEN
MOVE TO the dark red band, r 76–90

13 Psoas major

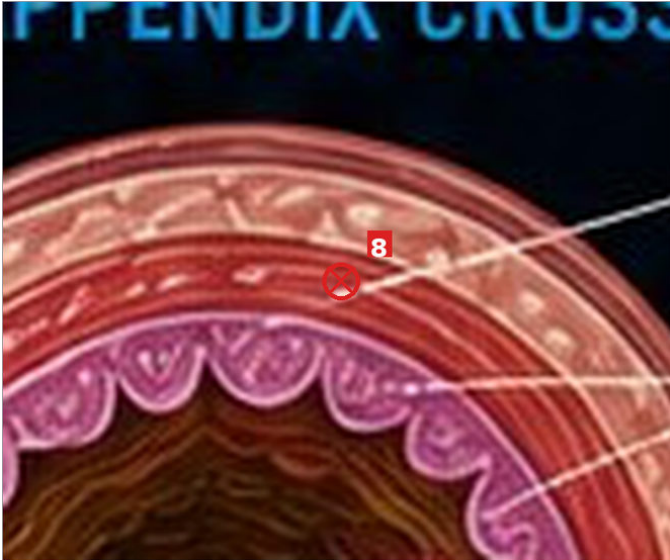
NOW (354,1108) the haustrated CECUM
(pink sacculated bowel, x 335–420)
MOVE TO the muscle — CONFIRM IT IS DRAWN;
we could not find separately drawn psoas
or iliacus bellies

14 Iliacus

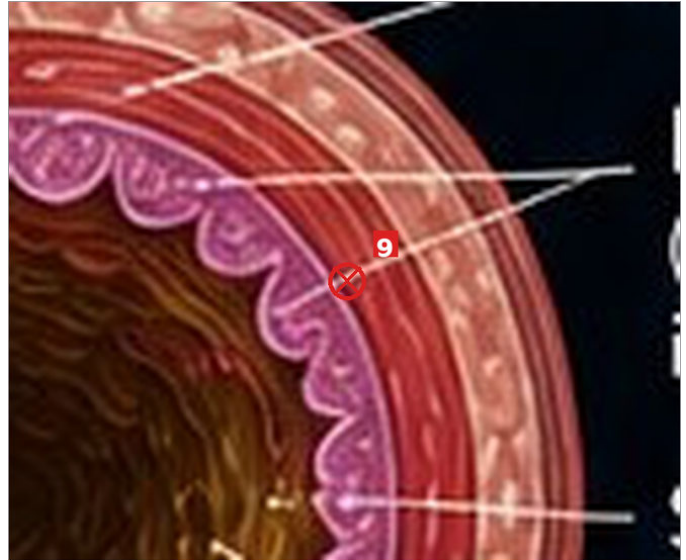
NOW (340,1152) the same CECUM, at its
left margin, 45 px below
MOVE TO the muscle — CONFIRM IT IS DRAWN

Appendicitis — page 2 — close-ups

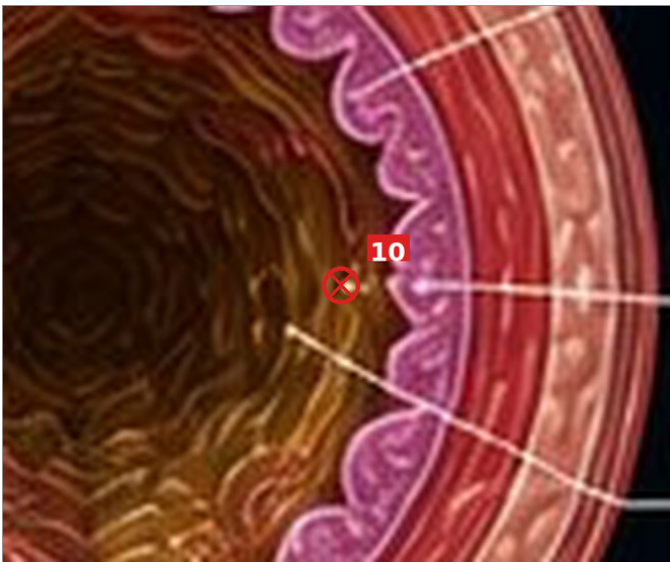
red crossed circle = where it ends now green plain circle = where it belongs



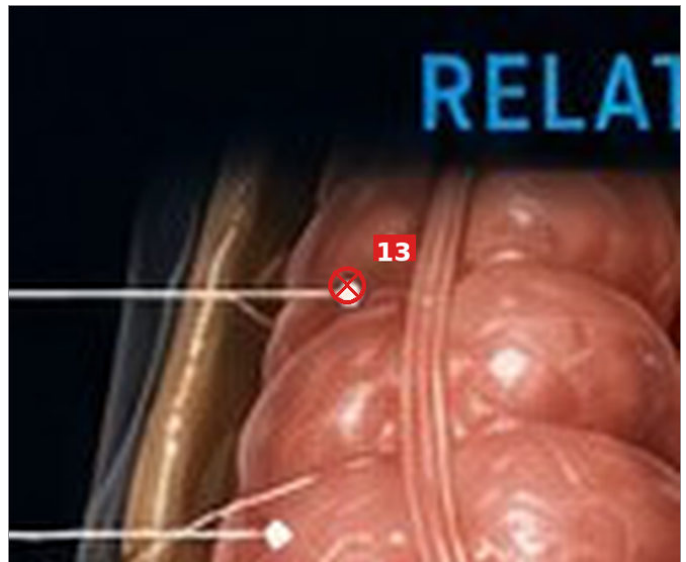
8 Serosa
(160,821) $r = 83$ — the dark red SUBMUCOSA
-> the outer rim, $r \approx 110$



9 Muscularis propria
(200,859) $r = 78$ — the mucosa/submucosa boundary
-> the pale speckled band, $r 91-108$



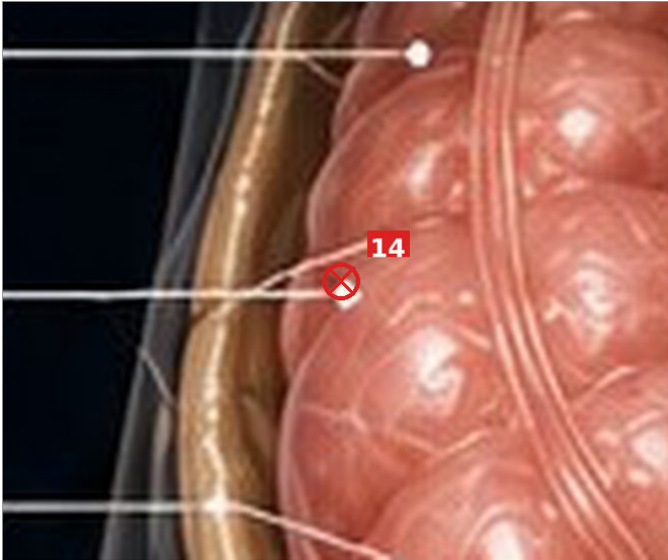
10 Submucosa
(185,900) $r = 51$ — the orange LUMEN
-> the dark red band, $r 76-90$



13 Psoas major
(354,1108) the haustrated CECUM (pink sacculated bowel, $x 335-420$)
-> the muscle — CONFIRM IT IS DRAWN; we could not find separately drawn psoas or iliacus bellies

Appendicitis — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



14 Iliacus

(340,1152) the same CECUM, at its left margin, 45 px below

-> the muscle — CONFIRM IT IS DRAWN

NOTES

A check that needs no anatomy: on this figure no two layer labels should terminate at the same radius, and the five radii should ascend lumen < mucosa < submucosa < muscularis < serosa. They do not. Rows 11 (Mucosa, on the lumen) and 12 (Lumen, on the magenta band) are carried at the order's readings. Rows 15 (Right ureter, on the mid-appendix) and 16 (External iliac vessels, on the distal appendix) matter alongside 13 and 14: all four retroperitoneal relations on that panel are labelled onto bowel or appendix.

CARRIED FROM THE WORK ORDER - NOT RE-MEASURED HERE

4 Mesappendix

two dots on the lateral wall of the appendix itself — and no peritoneal fold appears to be drawn, so this may need artwork

6 Appendicular vein

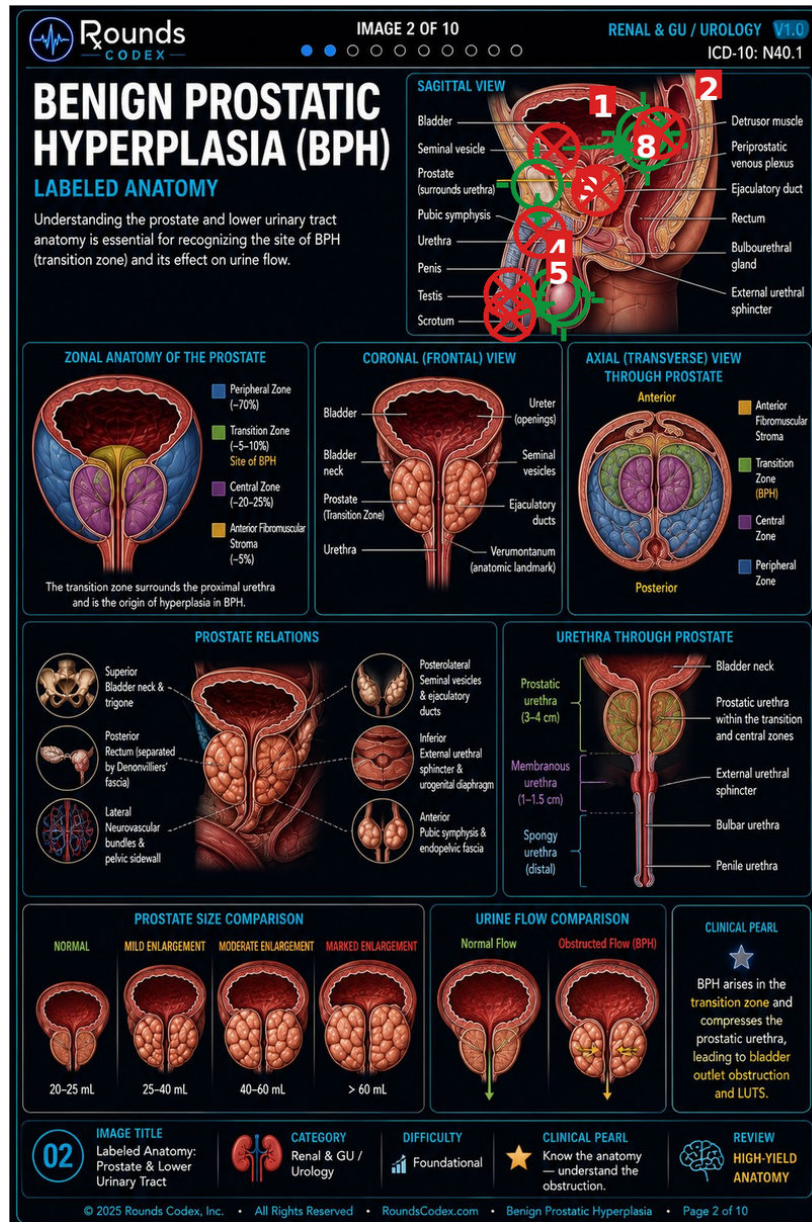
the top of the ileocolic venous trunk rather than the small vein running to the appendix

BPH — page 2

Sagittal male pelvis · bph-02.jpg · 1024×1536 px · 6 findings

The worst page in the order: 12 of 33 labels flagged. 6 re-measured at 9×; 6 carried.

Rows 1 and 2 are the reason this page is first in the queue: the seminal vesicle is labelled Detrusor muscle and the bladder wall is labelled Seminal vesicle. They approach from opposite sides, so it is not a swapped pair of leaders — each is displaced in a way that happens to land on the other's target. That is the failure mode which produces a page that LOOKS right.

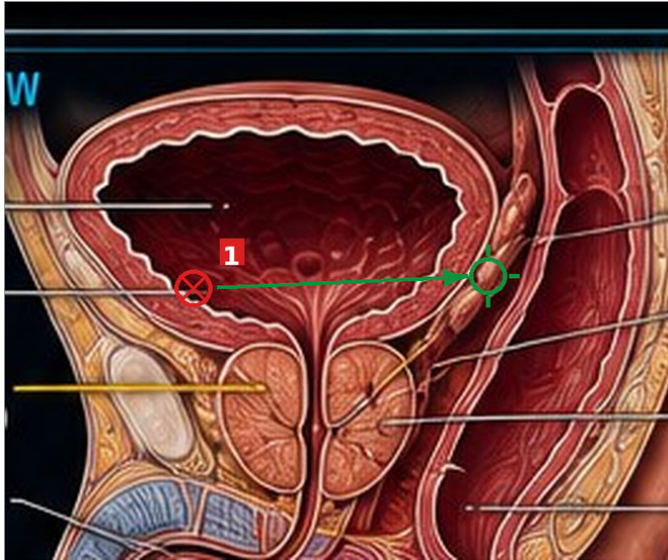


WHAT TO CHANGE

- Seminal vesicle**
NOW (683,180) the bladder's anterior-inferior wall at the base
MOVE TO (800,175) the lumpy coiled retrovesical chain
- Detrusor muscle**
NOW (816,160) fascia lateral to that retrovesical chain
MOVE TO (795,160) the thick muscular bladder wall, 10-25 px medial
- Pubic symphysis**
NOW (666,285) the ROOT OF THE PENIS
MOVE TO (662,225) the pale cream ovoid, x 645-680, y 200-250
- Testis**
NOW (628,362) the penile shaft near the corona
MOVE TO (683,362) the pink ovoid, x 665-700, y 335-390
- Scrotum**
NOW (628,390) the GLANS PENIS
MOVE TO (695,370) the scrotal sac around the testis, x 660-730, y 320-405
- Periprostatic venous plexus**
NOW (738,232) a dark dot INSIDE the right prostate lobe
MOVE TO the veins around/anterior to the gland, outside it

BPH — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



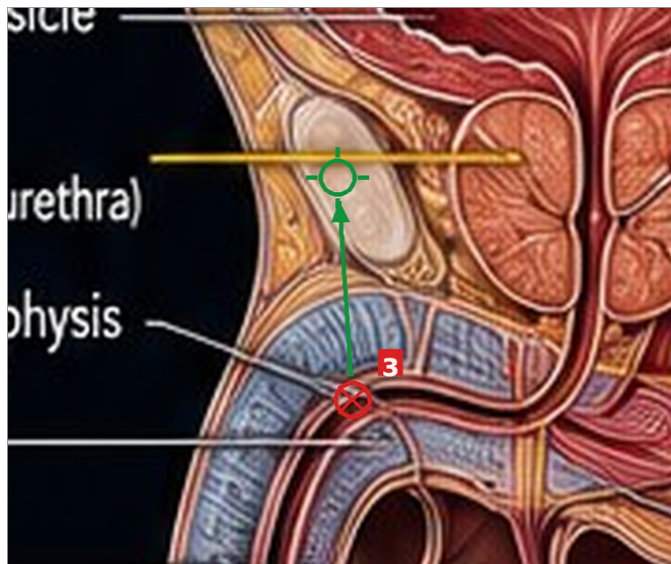
1 Seminal vesicle

(683,180) the bladder's anterior-inferior wall at the base
-> (800,175) the lumpy coiled retrovesical chain



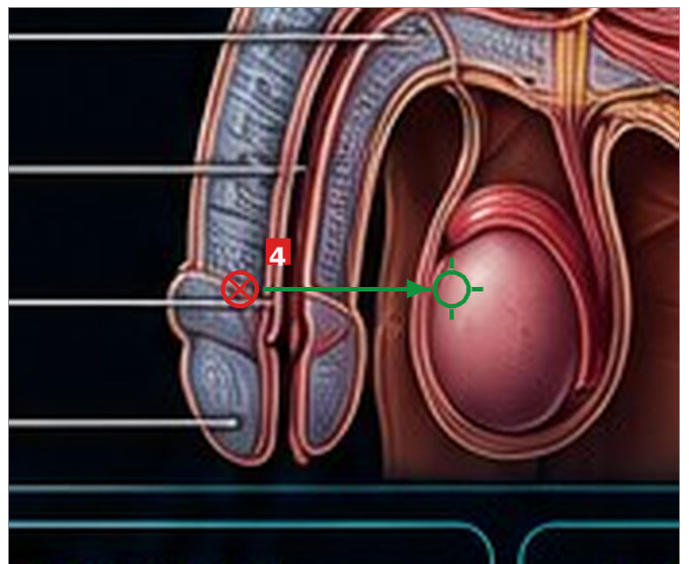
2 Detrusor muscle

(816,160) fascia lateral to that retrovesical chain
-> (795,160) the thick muscular bladder wall, 10–25 px medial



3 Pubic symphysis

(666,285) the ROOT OF THE PENIS
-> (662,225) the pale cream ovoid, x 645–680, y 200–250

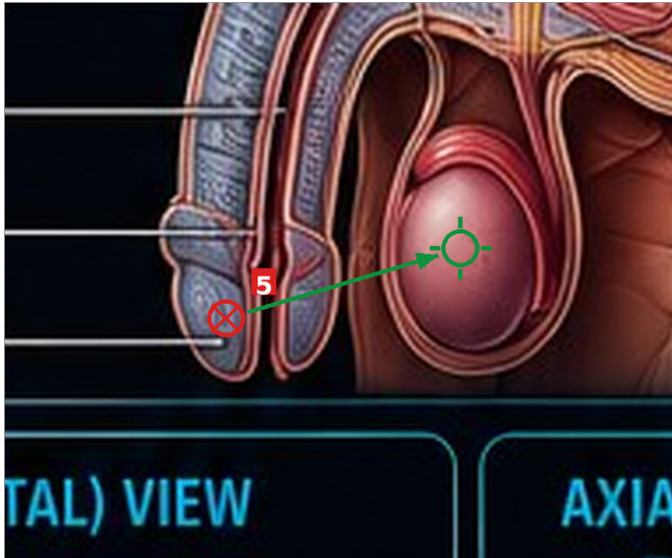


4 Testis

(628,362) the penile shaft near the corona
-> (683,362) the pink ovoid, x 665–700, y 335–390

BPH — page 2 — close-ups

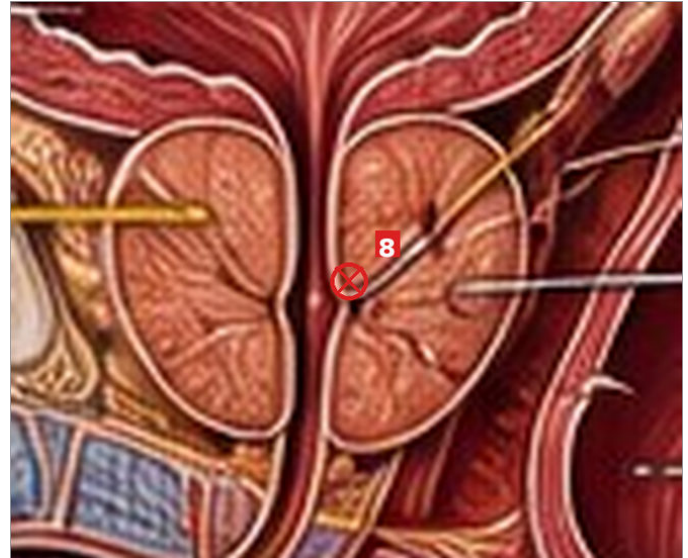
red crossed circle = where it ends now green plain circle = where it belongs



5 Scrotum

(628,390) the GLANS PENIS

-> (695,370) the scrotal sac around the testis, x
660–730, y 320–405



8 Periprostatic venous plexus

(738,232) a dark dot INSIDE the right prostate lobe

-> the veins around/anterior to the gland, outside it

NOTES

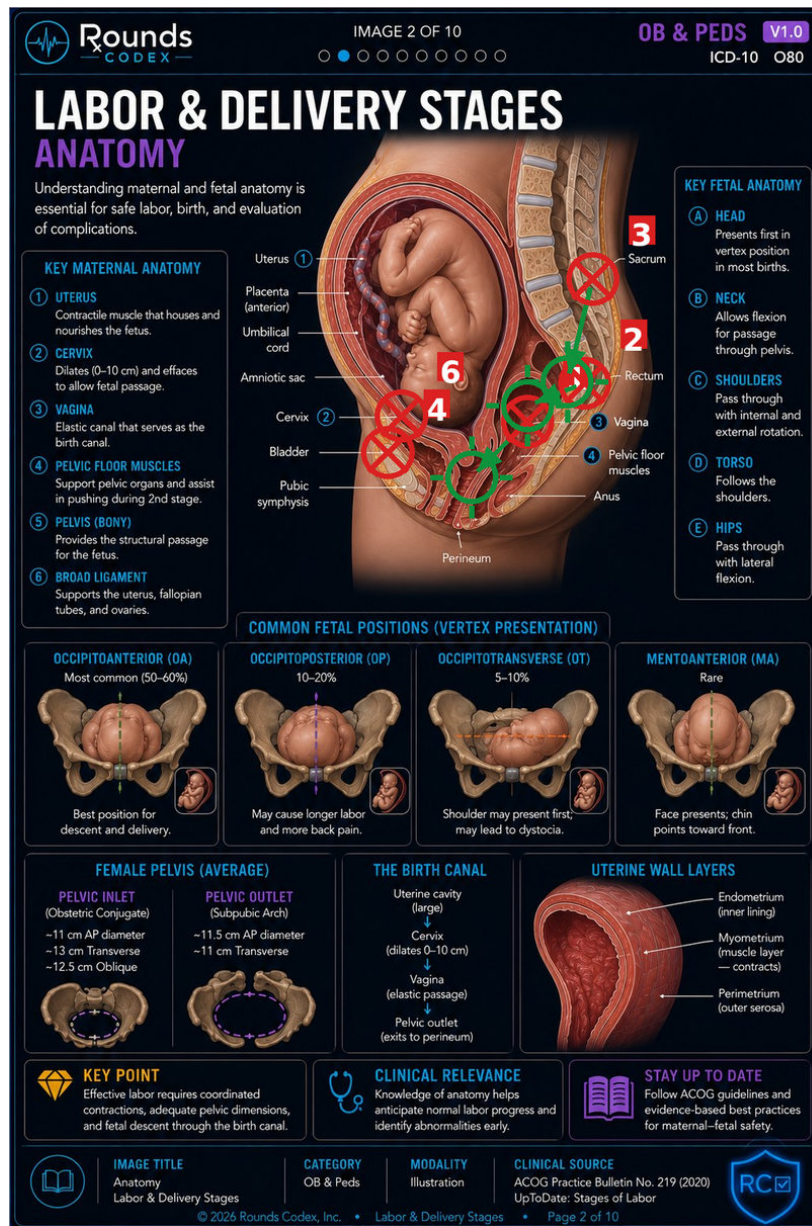
LAYOUT WARNING for rows 4 and 5. The label column runs down the left and the penis is drawn between that column and the testis, so a correct horizontal Testis leader would cross the glans and then the scrotal wall. Please re-site those two labels below the figure or bring them in from the right rather than lengthening the existing leaders. Also: the prostate's own extent is x 690–780, y 200–255, split at the midline urethra at x 728, with the pubic symphysis a separate pale ovoid at x 645–680 — at low magnification they read as one mass, which is very likely how Pubic symphysis ended up on the penile root.

Labor — page 2

Sagittal pelvis at term · labor-02.jpg · 1024×1536 px · 5 findings

5 of 7 measured and confirmed; 2 carried.

An obstetric anatomy page with the birth canal mislabelled. The right-hand column is displaced one position: Sacrum lands on a lumbar vertebra, Rectum lands on the sacrum, and Vagina lands inside the rectum.

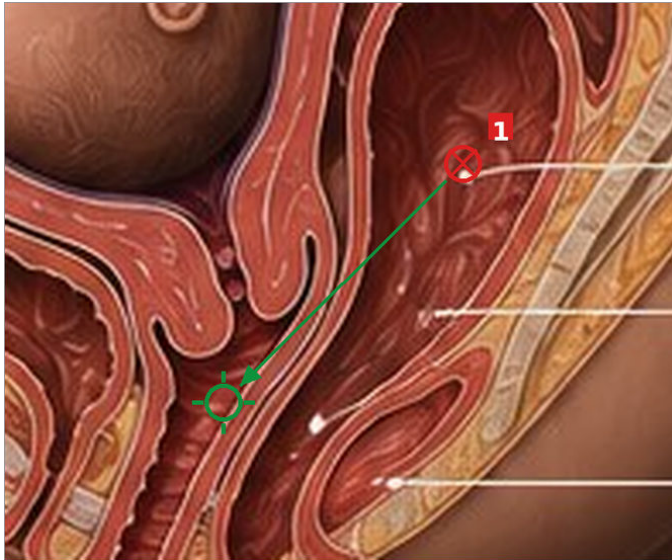


WHAT TO CHANGE

- Vagina ③**
NOW (650,520) inside the RECTUM
MOVE TO (580,590) the vaginal canal running down and forward from the cervix — PLACE FROM SOURCE
- Rectum**
NOW (722,472) the SACRUM
MOVE TO (650,500) the rectal lumen
- Sacrum**
NOW (732,341) the posterior elements of a LUMBAR vertebra
MOVE TO (700,470) the sacral body
- Bladder**
NOW (475,559) the PUBIC BONE
MOVE TO the bladder lumen — CONFIRM IT IS DRAWN
- Cervix ②**
NOW (492,513) the lower-segment wall
MOVE TO the cervical canal

Labor — page 2 — close-ups

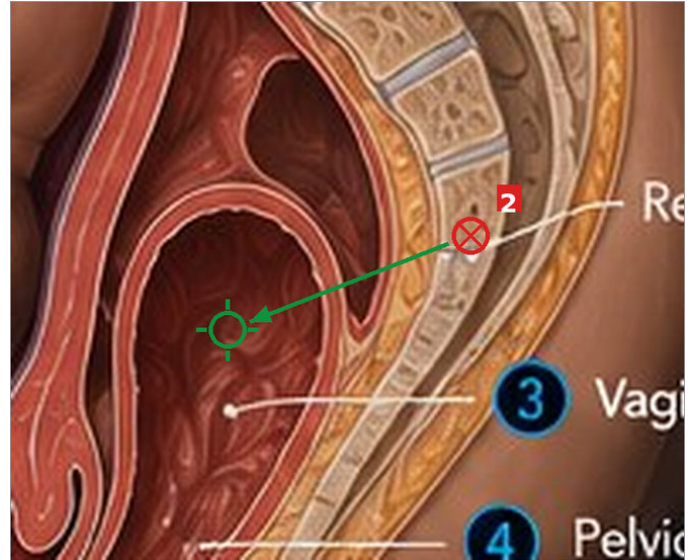
red crossed circle = where it ends now green plain circle = where it belongs



1 Vagina ③

(650,520) inside the RECTUM

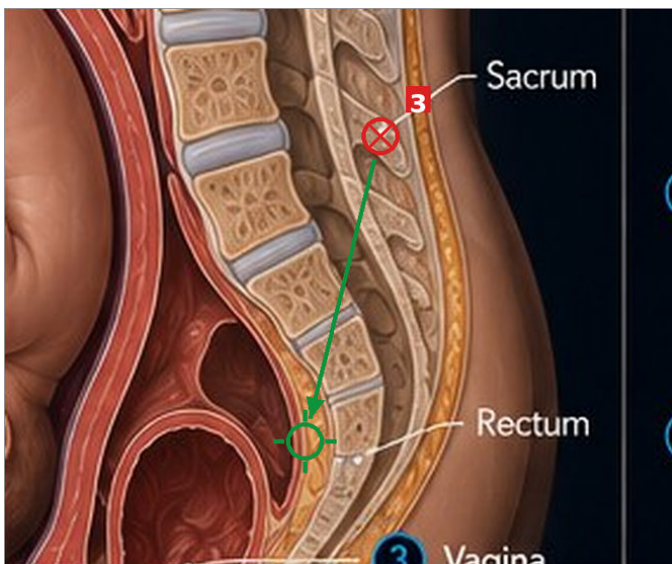
-> (580,590) the vaginal canal running down and forward from the cervix — PLACE FROM SOURCE



2 Rectum

(722,472) the SACRUM

-> (650,500) the rectal lumen



3 Sacrum

(732,341) the posterior elements of a LUMBAR vertebra

-> (700,470) the sacral body



4 Bladder

(475,559) the PUBIC BONE

-> the bladder lumen — CONFIRM IT IS DRAWN

Labor — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



6 Cervix ②

(492,513) the lower-segment wall

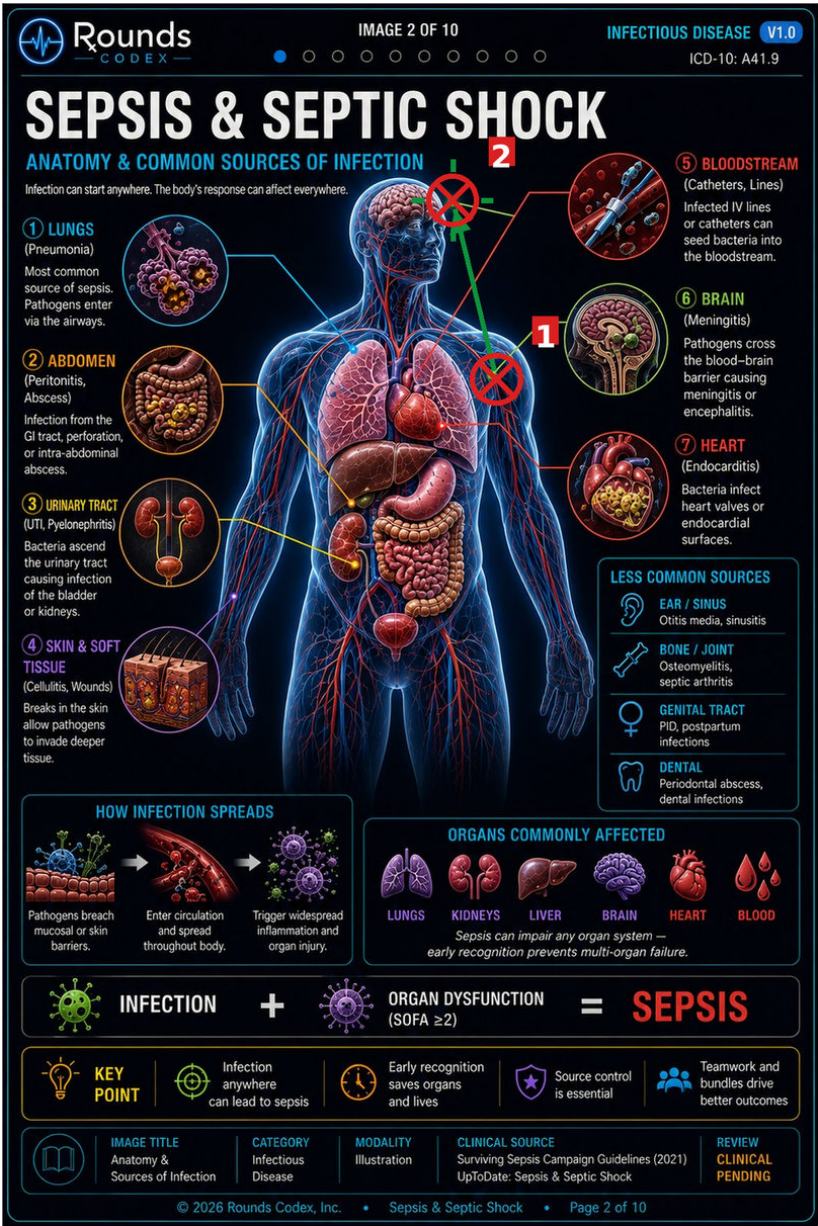
-> the cervical canal

Sepsis — page 2

Organ callouts · sepsis-02.jpg · 1024×1536 px · 2 findings

Confirmed. One of the two worst pages in the triage.

Callout ⑥ BRAIN (Meningitis) runs from the right column down-left and dies in the left upper chest, about 225 px below the head — while the correct green marker sits beside the temple, orphaned, its short stub dying where it crosses the red BLOODSTREAM leader.

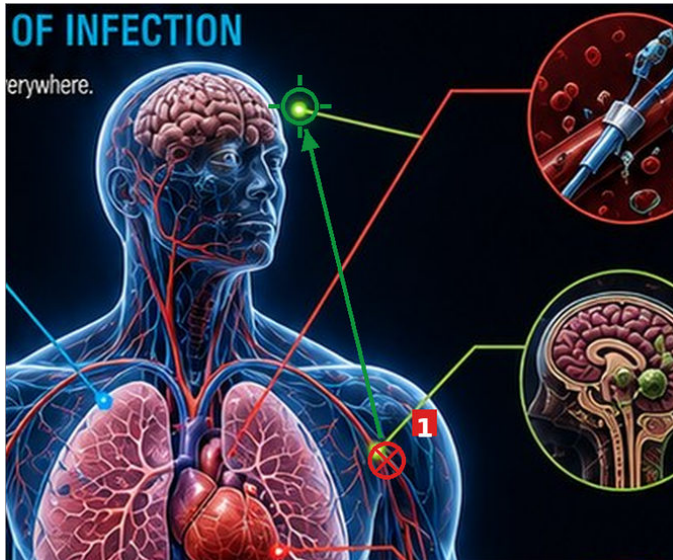


WHAT TO CHANGE

- 1 ⑥ BRAIN (Meningitis)
NOW (620,472) the left upper chest / lung region
MOVE TO (565,247) the orphaned green temple marker — the correct site
- 2 green temple marker
NOW (565,247) ORPHANED — its stub runs down-right and dies at (642,268), exactly where it crosses the red BLOODSTREAM leader
MOVE TO should receive the BRAIN leader

Sepsis — page 2 — close-ups

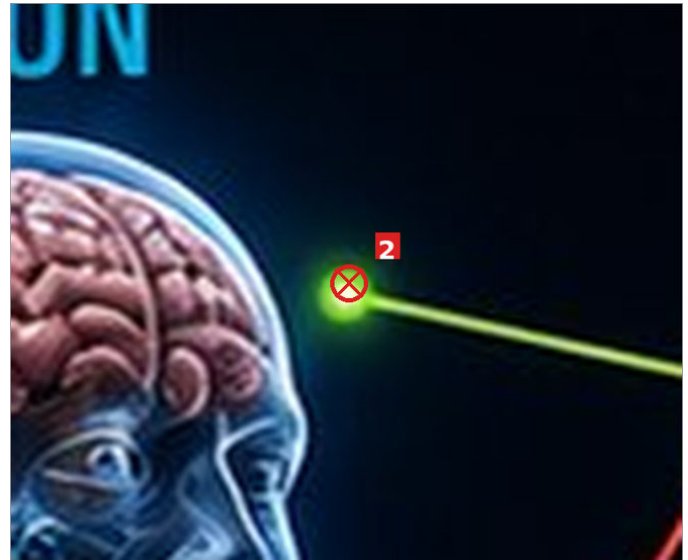
red crossed circle = where it ends now green plain circle = where it belongs



1 ⑥ BRAIN (Meningitis)

(620,472) the left upper chest / lung region

-> (565,247) the orphaned green temple marker — the correct site



2 green temple marker

(565,247) ORPHANED — its stub runs down-right and dies at (642,268), exactly where it crosses the red BLOODSTREAM leader

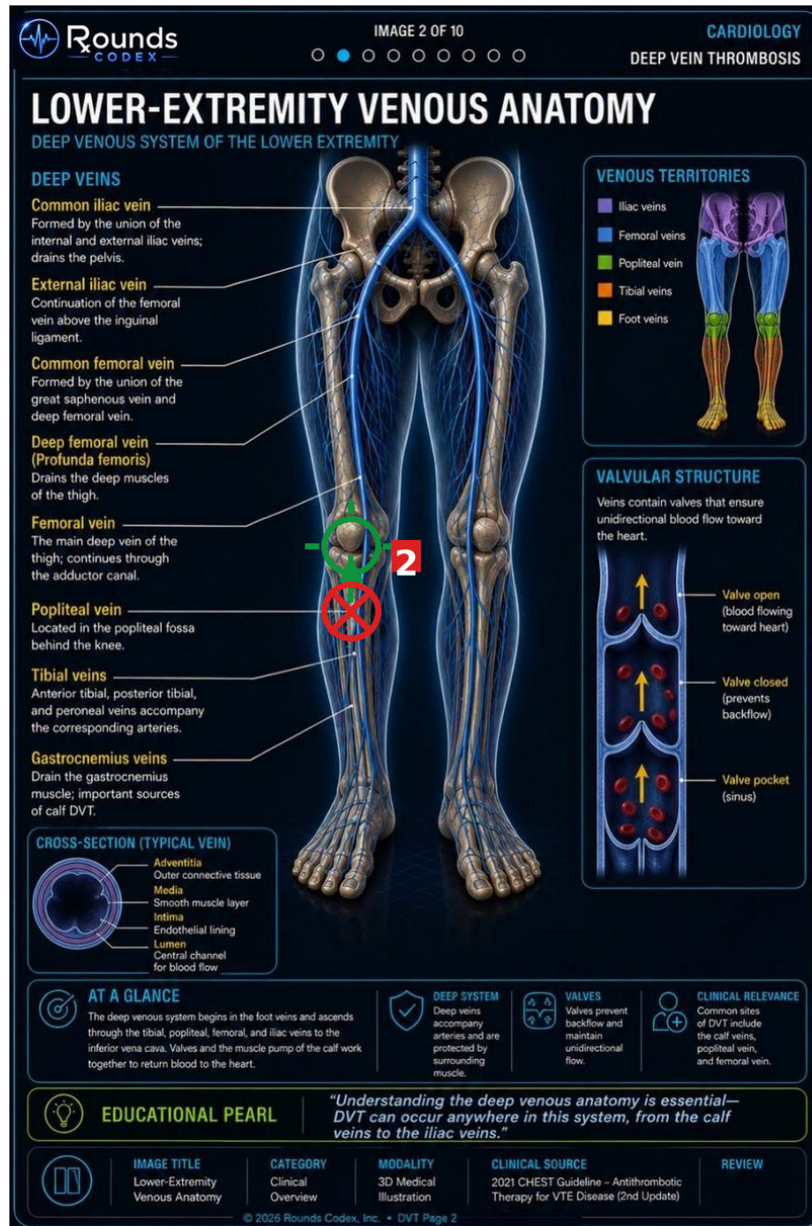
-> should receive the BRAIN leader

DVT — page 2

Deep venous anatomy of the leg · dvt-02.jpg · 913×1373 px · 1 findings

Confirmed. The other of the two worst pages in the triage.

The popliteal vein is, by definition, the vein behind the knee. Its label is on the tibia.



WHAT TO CHANGE

2 Popliteal vein

NOW (382,678) the proximal TIBIAL SHAFT,
73 px below the knee joint line
MOVE TO (382,605) the vein behind the
knee, at the joint line (y = 605)

DVT — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



2 Popliteal vein

(382,678) the proximal TIBIAL SHAFT, 73 px below the knee joint line

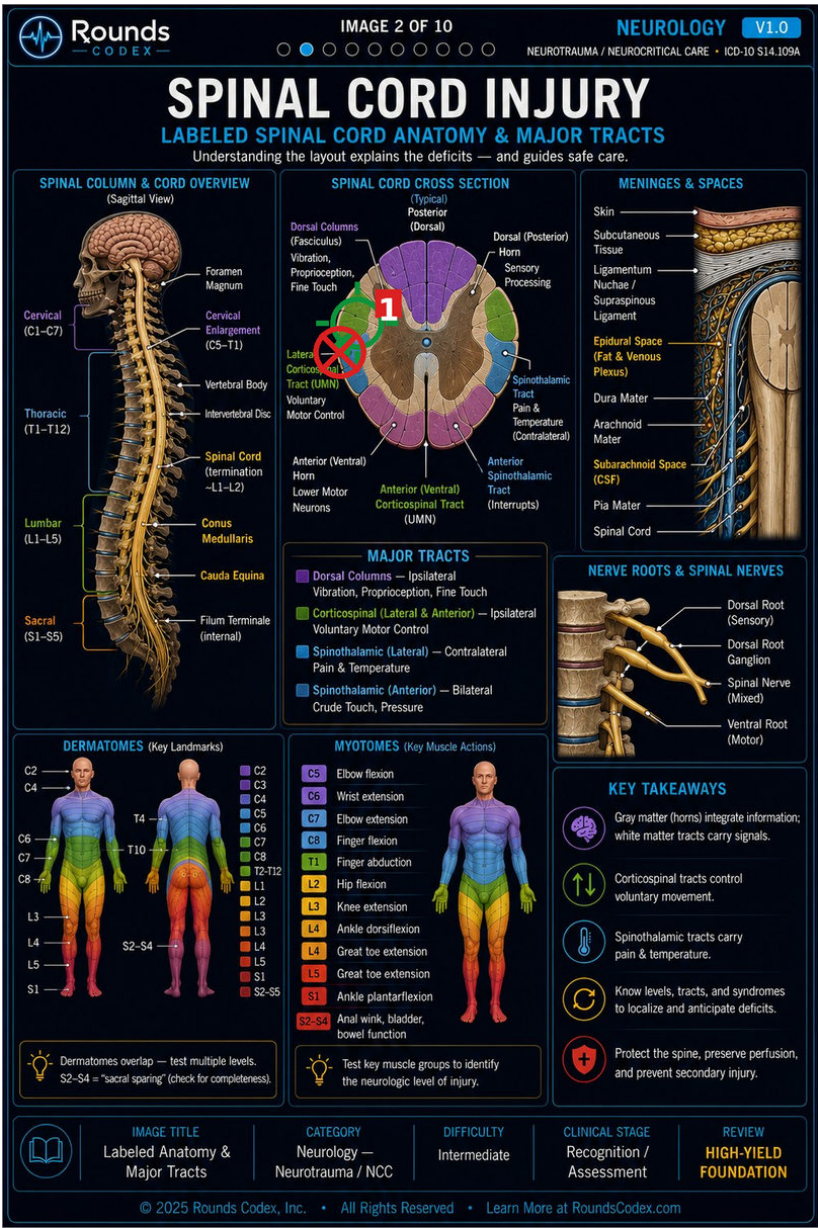
-> (382,605) the vein behind the knee, at the joint line
(y = 605)

Spinal cord injury — page 2

Major tracts (cord cross-section) · sci-02.jpg · 1024×1536 px · 1 findings

Confirmed by sampling the colour under the endpoint against the page's own key.

The page prints a MAJOR TRACTS key: Dorsal columns (86,39,140) purple, Corticospinal (66,110,24) green, Spinothalamic (27,118,181) blue. The label contradicts that key — no anatomical judgement is involved.

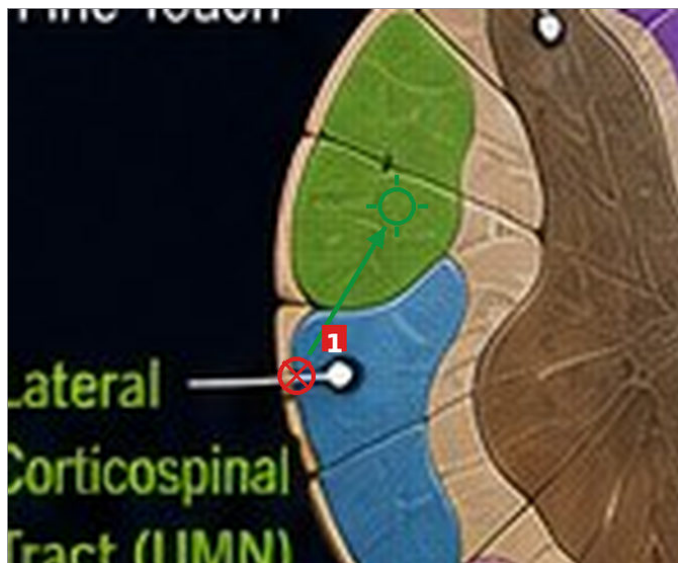


WHAT TO CHANGE

- 1 Lateral Corticospinal Tract (UMN)
NOW (423,437) the SPINOTHALAMIC blue
MOVE TO (445,400) the green
posterolateral region — drawn 30–45 px up
and left, sampled (100,136,43) and
(105,136,53), and currently carrying no
label

Spinal cord injury — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 Lateral Corticospinal Tract (UMN)

(423,437) the SPINOTHALAMIC blue

-> (445,400) the green posterolateral region — drawn
30–45 px up and left, sampled (100,136,43) and
(105,136,53), and currently carrying no label

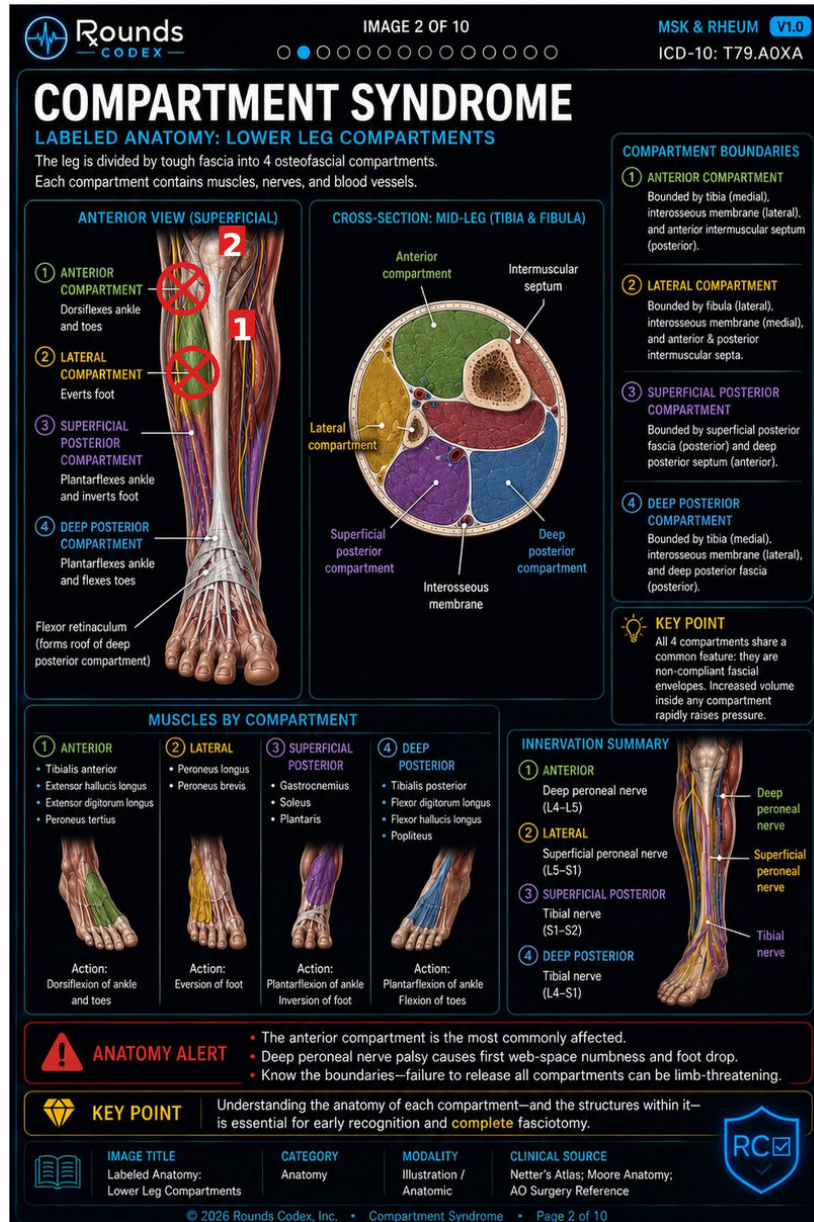
Leg compartments (cross-section) · compartment-02.jpg · 1024×1536 px · 2 findings

Confirmed by colour sampling against the page's own key.

Same method as sci p2: sample the colour under each endpoint, look it up in the key, compare with the label.

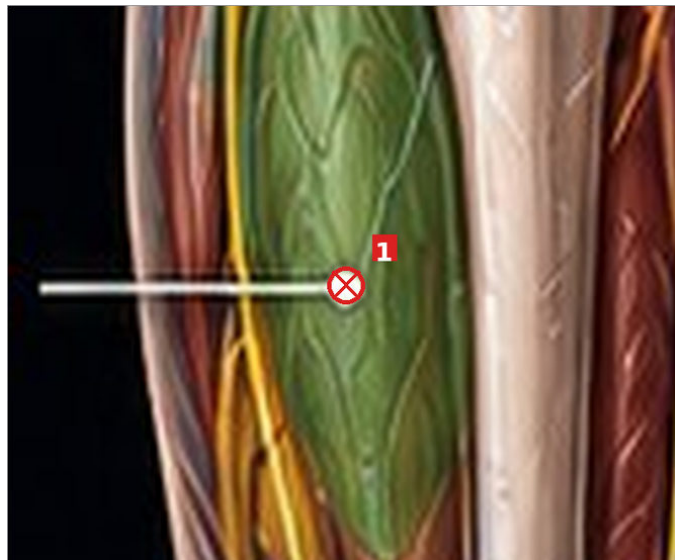
WHAT TO CHANGE

- 1 ② **LATERAL (gold)**
NOW (230,462) sampled (83,103,51) — the same mass as the belly at (235,410), (92,104,52)
MOVE TO the gold peroneal group over the fibula
- 2 **ANTERIOR (green)**
NOW (218,357) pinkish-tan muscle, sampled (177,154,137) — not the green compartment
MOVE TO the green anterior compartment



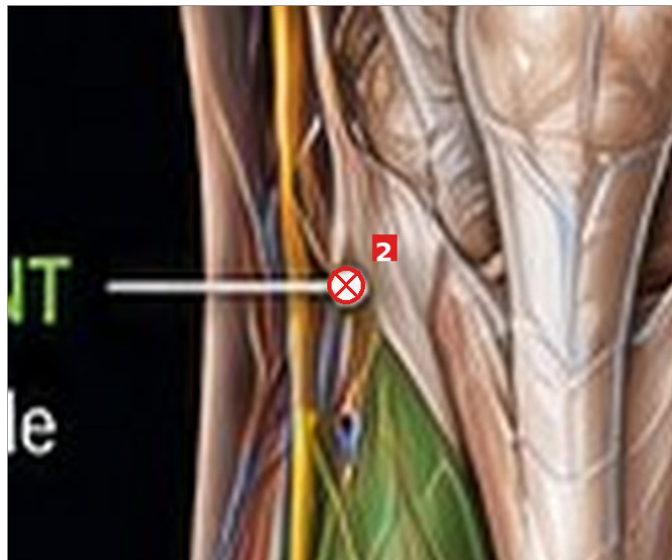
Compartment syndrome — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 ② LATERAL (gold)

(230,462) sampled (83,103,51) — the same mass as the belly at (235,410), (92,104,52)
-> the gold peroneal group over the fibula



2 ANTERIOR (green)

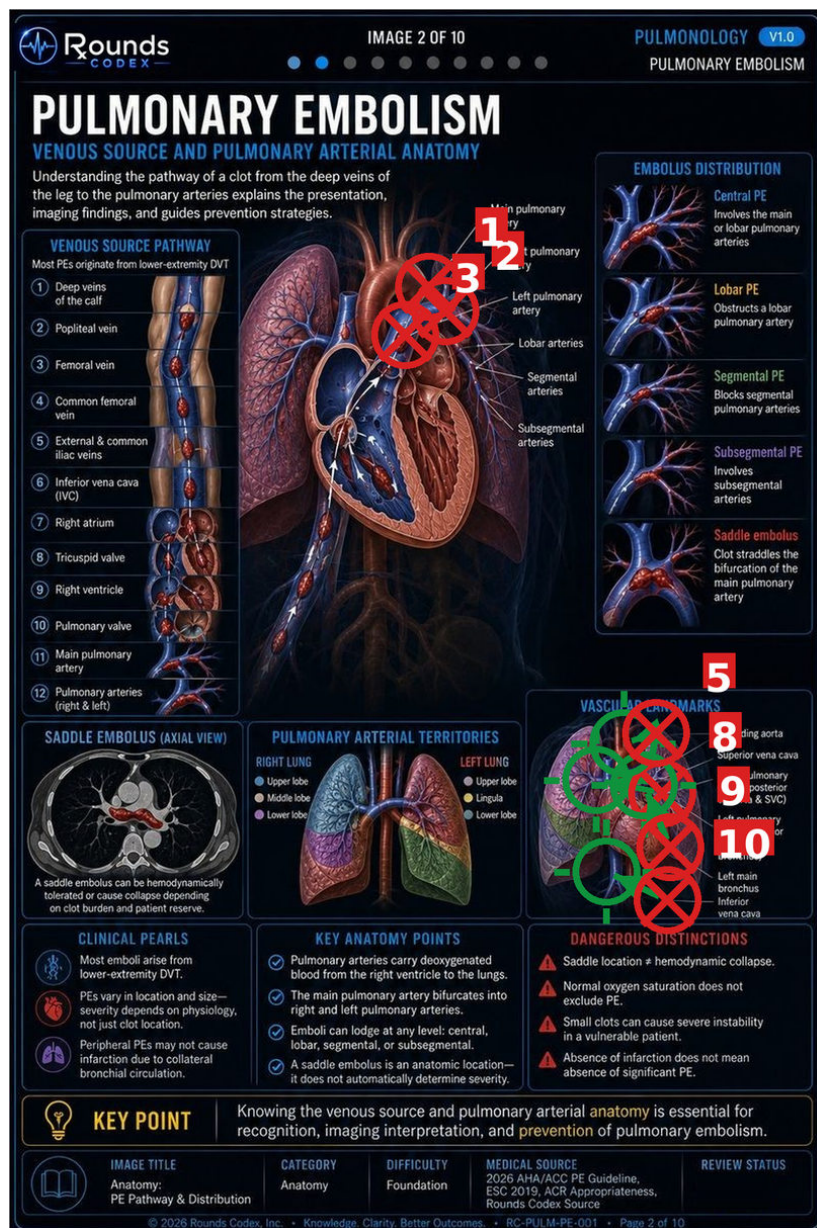
(218,357) pinkish-tan muscle, sampled (177,154,137) —
not the green compartment
-> the green anterior compartment

Pulmonary embolism — page 2

Central heart figure + vascular landmarks · pe-02.jpg · 800×1200 px · 8 findings

10 rows flagged; the pattern behind most of them is laterality. Page ships at 800×1200.

The side convention was established first: the aortic arch descends on the viewer's right (its limb runs down at x 400–430 while the ascending aorta rises at x 330–360), which on an anterior view puts the descending aorta on the patient's LEFT. Against that, 'Right pulmonary artery' points into the patient's left lung — twice, on two different panels.



WHAT TO CHANGE

- 2 Right pulmonary artery (heart figure)**
NOW (427,296) the blue artery running toward the VIEWER'S RIGHT — the patient's LEFT lung
MOVE TO the right pulmonary artery, viewer's left
- 1 Main pulmonary artery**
NOW (411,272) the right border of the salmon AORTIC ARCH — an artery of the systemic circulation
MOVE TO the blue pulmonary trunk
- 3 Left pulmonary artery [CHECK]**
NOW (388,319) the upper-right edge of the RED THROMBUS inside the blue trunk, rather than the vessel wall
MOVE TO the vessel wall
- 7 Right pulmonary artery (landmarks panel)**
NOW (640,769) the viewer's-right (patient's LEFT) hilum
MOVE TO (575,756) the right hilum, viewer's left, x 565–585, y 750–762
- 8 Left pulmonary artery [CHECK]**
NOW (640,769) THE SAME POINT as row 7 — two labels, one dot
MOVE TO its own point on the left PA
- 9 Left main bronchus**
NOW (648,822) left cardiac border / adjacent lung — NO AIRWAY IS DRAWN THERE
MOVE TO (622,761) the carina / left main bronchus behind the heart, x 615–630, y 755–768
- 5 Ascending aorta**
NOW (634,710) the APEX OF THE VIEWER'S-RIGHT LUNG; the leader fades out
MOVE TO (602,720) the salmon aorta, x 595–610, y 700–740
- 10 Inferior vena cava**
NOW (645,874) dark space where the leader fades
MOVE TO (585,846) the blue IVC, x 578–592, y 820–872

Pulmonary embolism — page 2 — close-ups

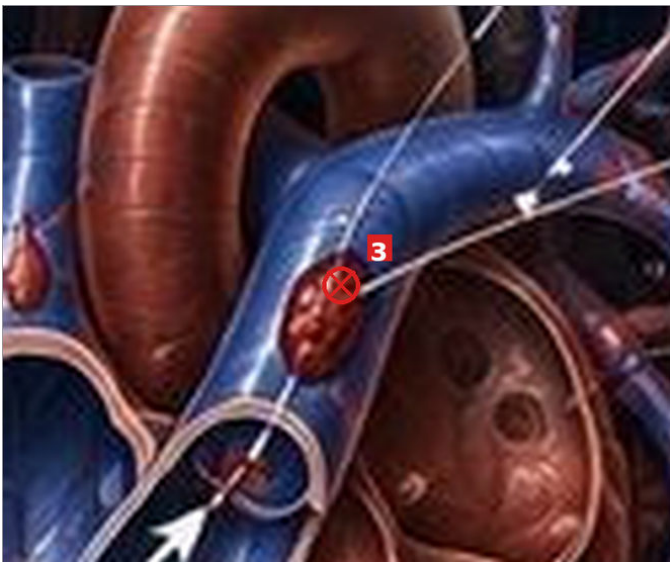
red crossed circle = where it ends now green plain circle = where it belongs



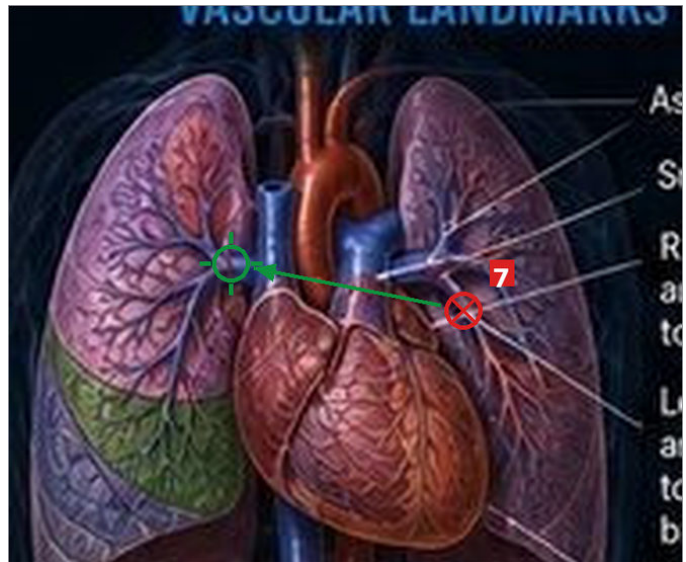
- 2 Right pulmonary artery (heart figure)**
 (427,296) the blue artery running toward the VIEWER'S RIGHT — the patient's LEFT lung
 -> the right pulmonary artery, viewer's left



- 1 Main pulmonary artery**
 (411,272) the right border of the salmon AORTIC ARCH — an artery of the systemic circulation
 -> the blue pulmonary trunk



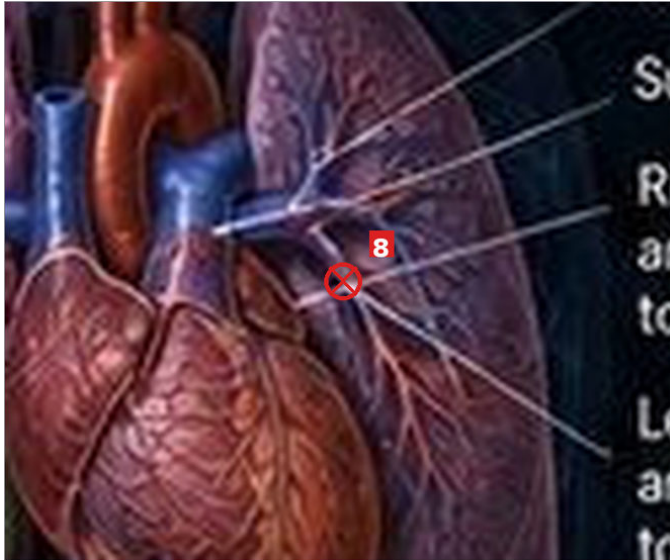
- 3 Left pulmonary artery [CHECK]**
 (388,319) the upper-right edge of the RED THROMBUS inside the blue trunk, rather than the vessel wall
 -> the vessel wall



- 7 Right pulmonary artery (landmarks panel)**
 (640,769) the viewer's-right (patient's LEFT) hilum
 -> (575,756) the right hilum, viewer's left, x 565–585, y 750–762

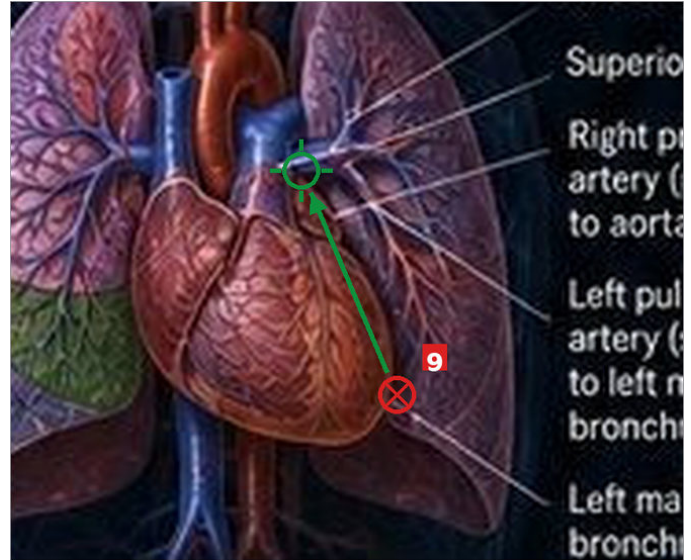
Pulmonary embolism — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



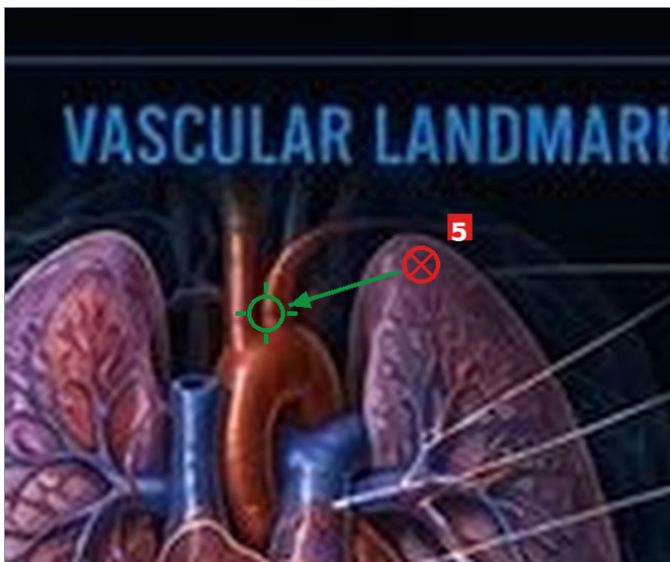
8 Left pulmonary artery [CHECK]

(640,769) THE SAME POINT as row 7 — two labels, one dot
-> its own point on the left PA



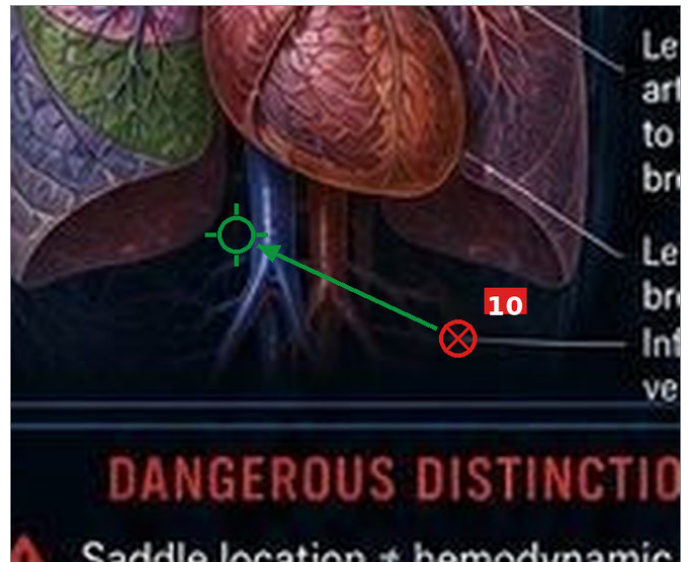
9 Left main bronchus

(648,822) left cardiac border / adjacent lung — NO
AIRWAY IS DRAWN THERE
-> (622,761) the carina / left main bronchus behind the
heart, x 615–630, y 755–768



5 Ascending aorta

(634,710) the APEX OF THE VIEWER'S-RIGHT LUNG; the
leader fades out
-> (602,720) the salmon aorta, x 595–610, y 700–740



10 Inferior vena cava

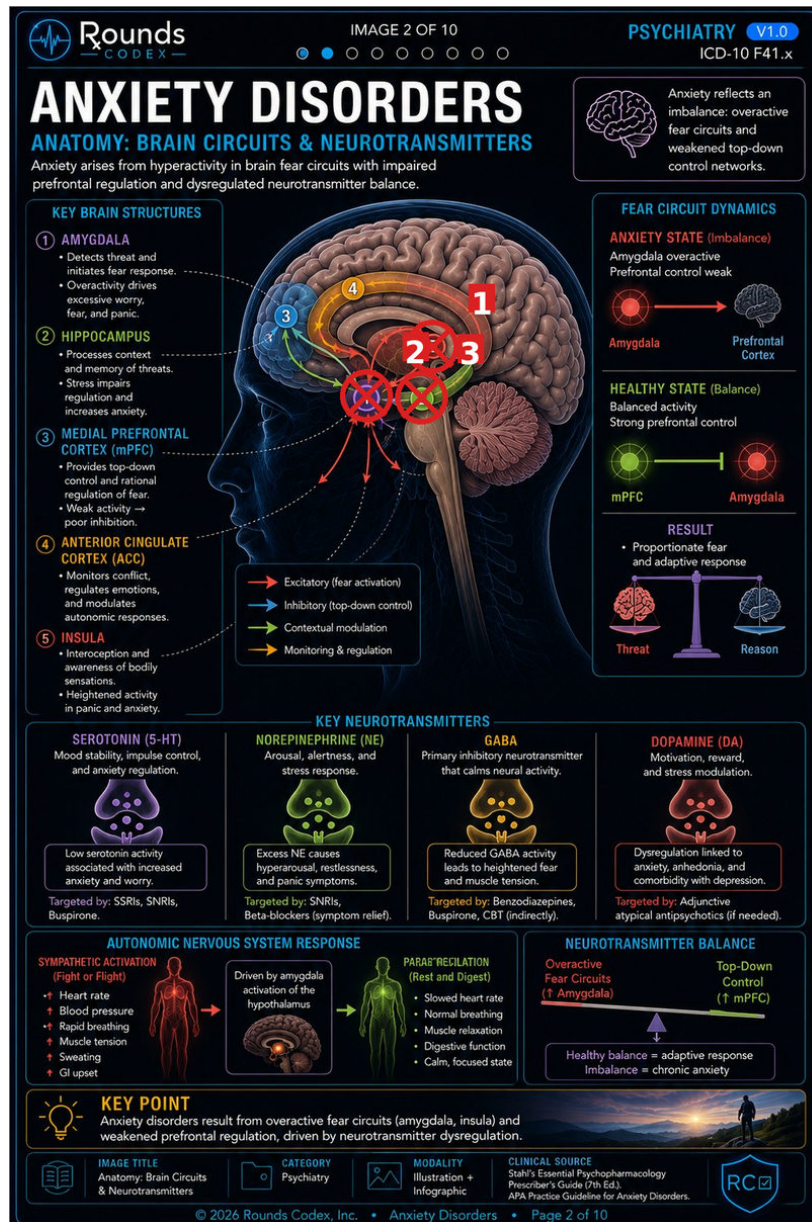
(645,874) dark space where the leader fades
-> (585,846) the blue IVC, x 578–592, y 820–872

Anxiety — page 2 [NEW, 2026-08-16]

Key brain structures — circuit figure · anxiety-02.jpg · 1024×1536 px · 3 findings

The order flagged 1 row. Measured and confirmed — and the panel sweep found TWO MORE of the same fault the order did not list.

The base image is a MIDSAGITTAL section — established from the artwork itself: the corpus callosum in full arc (x 370–600, y 340–430), the thalamus as a cut oval (x 452–565, y 404–446), the brainstem in section, the cerebellum with its arbor vitae. The amygdala, hippocampus and insula are all lateral to that plane. None of them can appear on this image, and none is drawn.



WHAT TO CHANGE

1 ⑤ INSULA

NOW (531,427) INSIDE THE DRAWN THALAMUS, at its upper-posterior part
MOVE TO not depictable on this cut — needs a lateral inset or a different base image

2 ① AMYGDALA (not in the work order)

NOW (448,489) below and anterior to the thalamus, at hypothalamic level
MOVE TO off-plane — same fault

3 ② HIPPOCAMPUS (not in the work order)

NOW (517,491) at the midbrain / cerebral peduncle, anterior to the cerebellum
MOVE TO off-plane — same fault

Anxiety — page 2 [NEW, 2026-08-16] — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



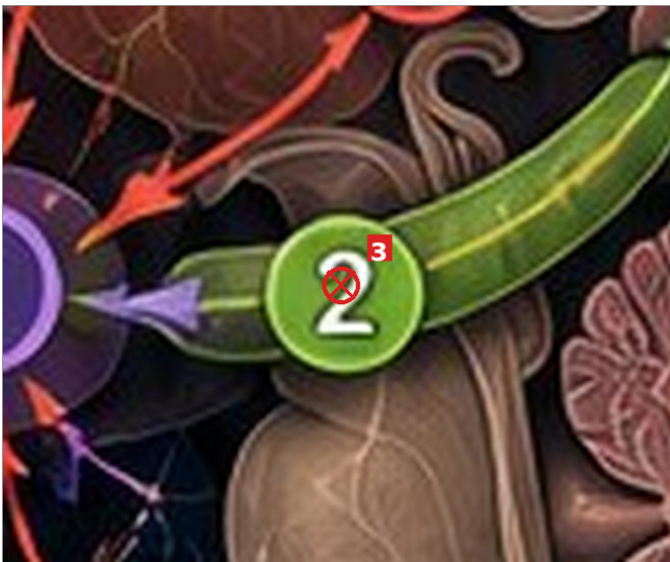
1 ⑤ INSULA

(531,427) INSIDE THE DRAWN THALAMUS, at its upper-posterior part
-> not depictable on this cut — needs a lateral inset or a different base image



2 ① AMYGDALA (not in the work order)

(448,489) below and anterior to the thalamus, at hypothalamic level
-> off-plane — same fault



3 ② HIPPOCAMPUS (not in the work order)

(517,491) at the midbrain / cerebral peduncle, anterior to the cerebellum
-> off-plane — same fault

Anxiety — page 2 [NEW, 2026-08-16] — notes

NOTES

IF ONLY ONE THING IS CHANGED, CHANGE ⑤. Rows ① and ② sit on tissue the page never names, which is merely imprecise. ⑤ sits on a drawn, recognisable thalamus that the page leaves unlabelled, so the reader is shown the thalamus and told it is the insula. Preferred fix: a small lateral or oblique inset carrying the three off-plane structures, leaving the main figure its circuit arrows and its two correct nodes (③ mPFC at (346,388) and ④ ACC at (432,354), both verified correct).

Airway cross-section, bronchiolitis side · bronchiolitis-07.jpg · 1024×1536 px · 1 findings

The circle draws three things unmistakably: the yellow mucus plug filling the centre (x 795–856, y 398–452), the pink oedematous wall from x 868 outward, and between them a dark residual air space — which is the narrowed lumen the label names. As drawn, the plug is labelled twice and the lumen not at all.

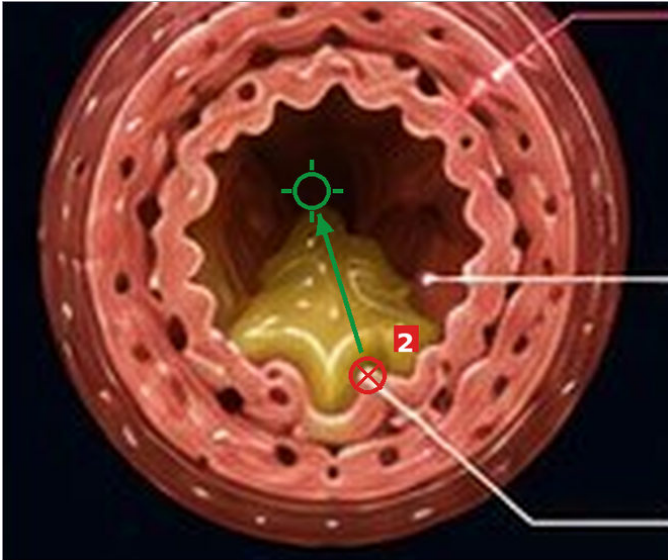
2 Narrowed lumen

NOW (845,443) the plug's inferior lobe
where it meets the wall, sampled
(192,145,106) — pale, not air
MOVE TO (832,400) the dark residual air
space, sampled (29,9,4). Also
(836,395)=(26,8,3) and (828,405)=(47,19,4)

Rounds Codex — mislabelled anatomy corrections

Bronchiolitis — page 7 [NEW, 2026-08-16] — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



2 Narrowed lumen

(845,443) the plug's inferior lobe where it meets the wall, sampled (192,145,106) — pale, not air
-> (832,400) the dark residual air space, sampled (29,9,4). Also (836,395)=(26,8,3) and (828,405)=(47,19,4)

NOTES

Row 1 (Minimal mucus, normal side) was corrected in-app in v95 and is verified fixed — no action.
Optional tidy while the file is open: the Mucus plug leader tip at (858,420) sits about 2 px beyond the plug's edge (the yellow ends at x 856); pull it back to (852,420) so the two leaders sit on opposite sides of the same boundary cleanly.

ACS — page 2

Coronary anatomy (numbered badges) · acs-02.jpg · 1138x1707 px · 1 findings

Not in the 81-page work order. Cited in the brief as the counter-example to 'badges are self-checking'.

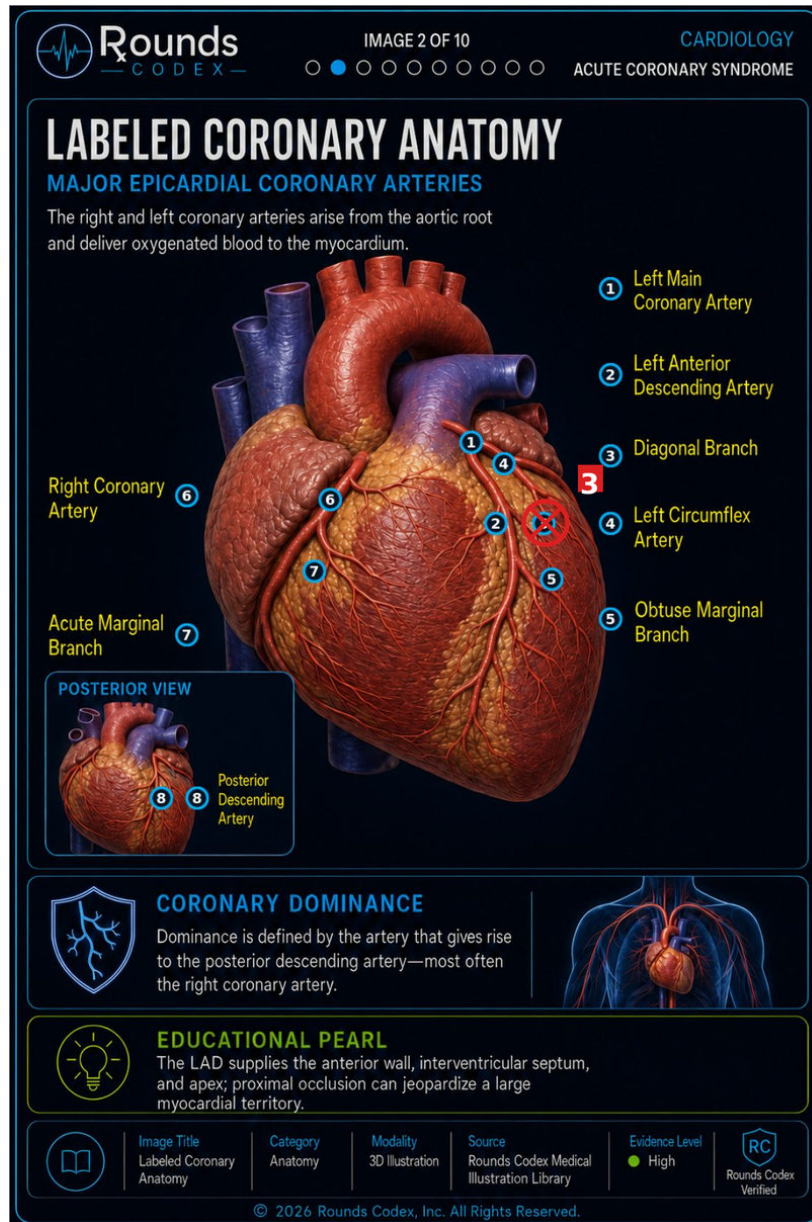
The bifurcation sits at (655,615). From it the LAD descends steeply through (672,660), (680,690), (690,722), (700,755); the circumflex runs right along (690,632) $\rightarrow$ (730,645) $\rightarrow$ (750,655). A diagonal branch arises from the LAD — that is what the name means, and it is why the badge's position matters.

WHAT TO CHANGE

3 ③ Diagonal Branch

NOW (748,722) a branch arising from the LEFT CIRCUMFLEX at about (712,640), descending through (727,690), (734,712), (740,735). At that height the LAD is at $x \approx 690$ — some 55 px left, with the badge's own edge still 45 px short of it

MOVE TO a branch arising from the LAD



ACS — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



3 ③ Diagonal Branch

(748,722) a branch arising from the LEFT CIRCUMFLEX at about (712,640), descending through (727,690), (734,712), (740,735). At that height the LAD is at $x \approx 690$ — some 55 px left, with the badge's own edge still 45 px short of it

-> a branch arising from the LAD

NOTES

Badge ⑤ (Obtuse Marginal) sits roughly 75 px further down the SAME vessel, so two badges name different vessels on one branch — re-check ⑤ once ③ is no longer sharing it. METHOD NOTE: the branch was traced by eye at 11× on the shipped JPEG, not measured. We tried to isolate the vessels numerically and could not — on this render the myocardium is nearly as red as the arteries. You have the layers; if the trace disagrees with your source file, your source file is right.

ACS — page 6

Magnifier ring on the anterior heart · acs-06.jpg · 1138×1707 px · 1 findings

Measured. Needs a re-render — see the note.

The ring is centred at (761.6, 360.7), radius ~10.7 px. At 16× it contains NO coronary artery at all, only myocardial surface texture — while the inset it feeds shows an atherosclerotic plaque inside a coronary artery.

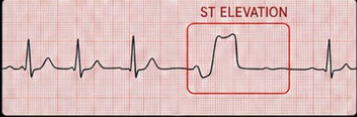
ACUTE CORONARY SYNDROME

DIAGNOSTIC IMAGING & TESTING

Diagnosis integrates symptoms, serial ECGs, cardiac troponin, and targeted imaging.

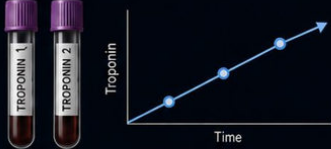
1 12-LEAD ECG

Obtain promptly and repeat when symptoms or clinical suspicion persist. Identify ST elevation, ST depression, T-wave inversion, or arrhythmia.



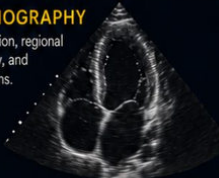
2 HIGH-SENSITIVITY TROPONIN

Use serial measurements to detect a rise or fall consistent with myocardial injury.



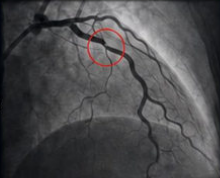
3 ECHOCARDIOGRAPHY

Assess ventricular function, regional wall-motion abnormality, and mechanical complications.



4 CORONARY ANGIOGRAPHY

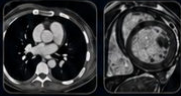
Defines culprit coronary anatomy and permits immediate PCI when indicated.





CHEST RADIOGRAPHY

Evaluates pulmonary edema and alternative thoracic causes.



CT OR CMR

Reserved for selected patients when diagnosis or anatomy remains uncertain.



STEMI PRIORITY

Do not delay indicated reperfusion for nonessential imaging.



INTEGRATED DIAGNOSTIC APPROACH

Combine clinical assessment, serial ECGs and troponin with targeted imaging to define diagnosis, risk, and treatment.



EDUCATIONAL PEARL

A normal initial ECG or troponin does not exclude ACS. Repeat testing when suspicion remains high.

Image Title: Diagnostic Imaging & Testing

Category: Diagnostic Evaluation

Modality: Clinical Imaging & Graphics

Source: Rounds Codex Medical Illustration Library

Evidence Level: High

Verification: Rounds Codex Verified

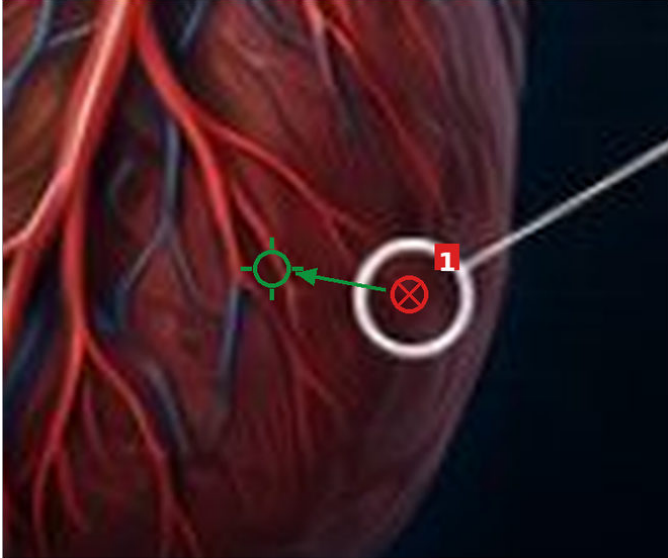
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WHAT TO CHANGE

- 1 magnifier ring
NOW (762,361) bare myocardium — 5% of pixels inside the disc are vessel (mean R-G 33.0)
MOVE TO (735,356) 28 px to its left: 56% vessel (mean R-G 61.5). Candidates (712,350) and (724,372) score 20% and 11%

ACS — page 6 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 magnifier ring

(762,361) bare myocardium — 5% of pixels inside the disc
are vessel (mean R-G 33.0)
-> (735,356) 28 px to its left: 56% vessel (mean R-G
61.5). Candidates (712,350) and (724,372) score 20% and
11%

NOTES

The connector to the inset then runs from the new ring edge to the same point on the inset circle, (828,322). ACS page 3 does this correctly, so it is a placement slip rather than a limitation of the layout. WHY NOT FIXED LOCALLY: this needs a 28 px disc of shaded myocardium erased near the heart's silhouette edge; four clone offsets were rendered and every one left a visible pale patch on the gallery's hero image.

Aortic stenosis — page 2

Cut heart · aortic-stenosis-02.jpg · 1138×1707 px · 1 findings

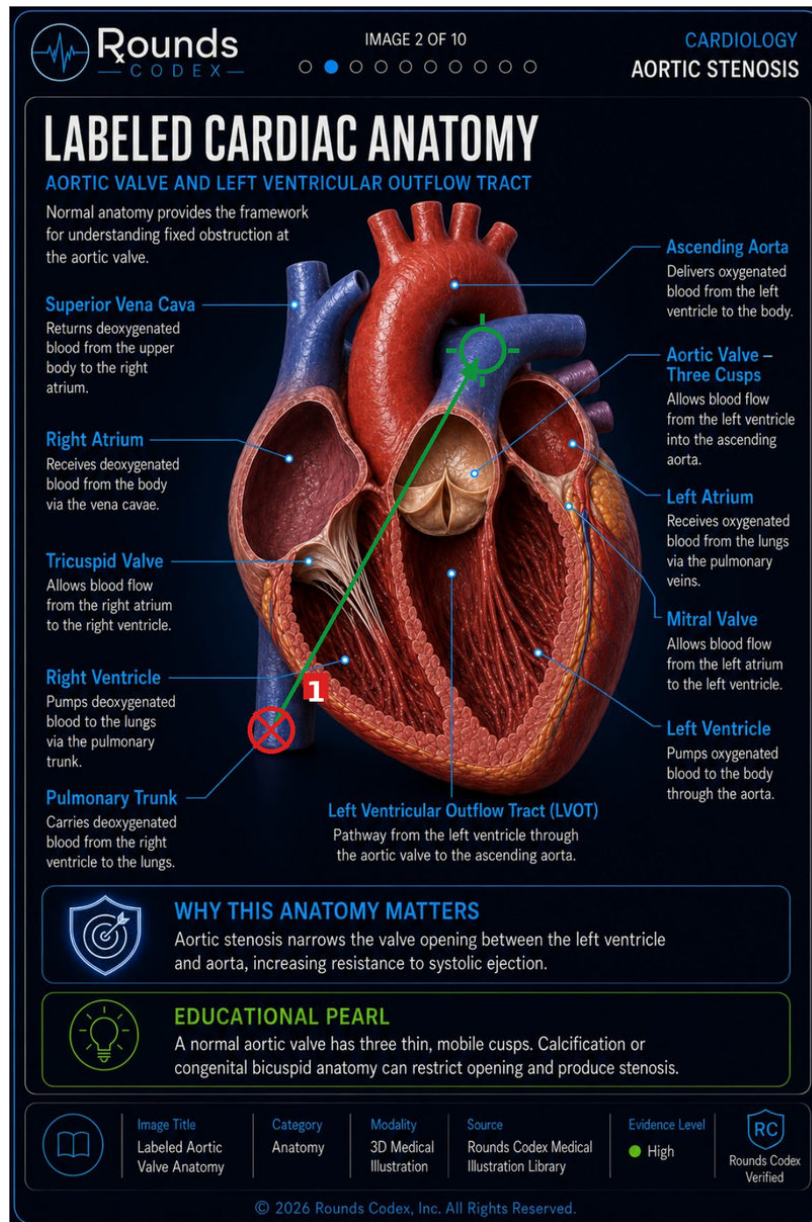
A LAYOUT change, not a leader correction.

'Pulmonary Trunk — carries deoxygenated blood from the right ventricle to the lungs' ends on the large blue vessel that descends behind the right atrium and leaves the bottom of the frame. That vessel is the INFERIOR VENA CAVA, which carries blood INTO the right atrium — the opposite direction from the one the caption describes. The page labels the SVC correctly three inches above it, so a reader has both cavae in view with one of them named as an outflow vessel.

WHAT TO CHANGE

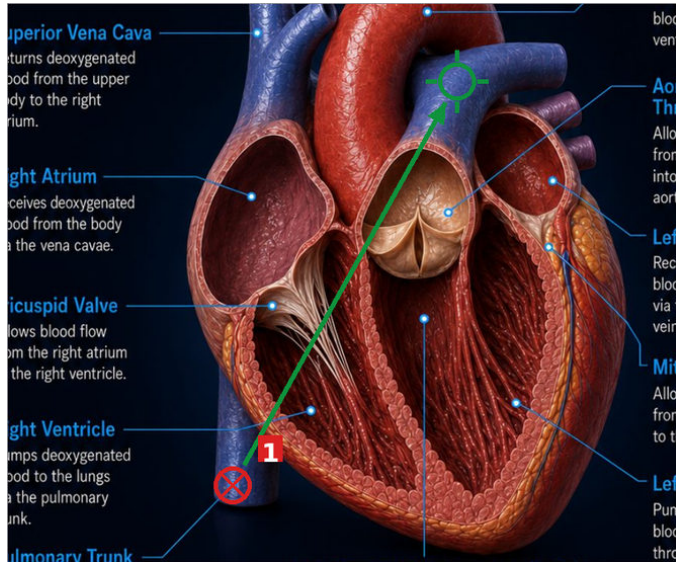
1 Pulmonary Trunk

NOW (365,1010) the inferior vena cava
MOVE TO (660,480) the blue trunk at its
mid-course, drawn at x 560–800, y 420–560,
arising anterior to the aortic root —
currently carrying no label at all



Aortic stenosis — page 2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 Pulmonary Trunk

(365,1010) the inferior vena cava

-> (660,480) the blue trunk at its mid-course, drawn at
x 560–800, y 420–560, arising anterior to the aortic
root — currently carrying no label at all

NOTES

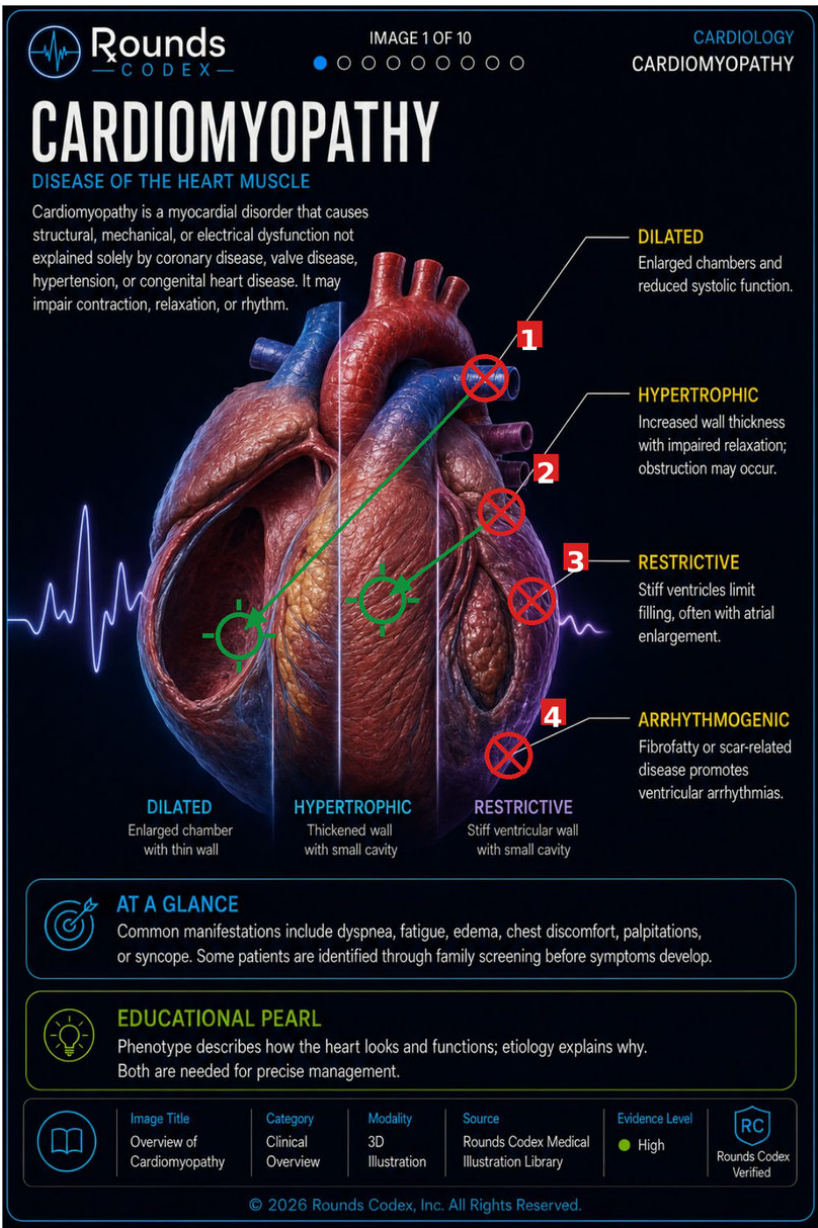
DO NOT SIMPLY RE-AIM THE EXISTING LEADER: from its present position the line would cross the right atrium, the tricuspid valve, the right ventricle and the LVOT leaders. Re-site the label block to the upper left or upper centre and run a short leader. Two consequences to decide at the same time: (1) the vessel at (365,1010) is then unlabelled — it is the IVC and the page already labels the SVC, so labelling it completes the pair and costs one line (recommended); (2) if the block cannot move, drop the leader and caption the trunk on-figure. Why the artist's arrangement made this easy to get wrong: SVC, IVC and pulmonary trunk are all rendered in the same blue, and two of the three are vertical tubes at the edge of the figure.

Cardiomyopathy — page 1

One heart split into three phenotype slabs · cardiomyopathy-01.jpg · 1138×1707 px · 4 findings

Three of four callouts point at the wrong phenotype.

The figure is one heart split by two vertical dividers into three slabs captioned DILATED | HYPERTROPHIC | RESTRICTIVE. Measured from the caption text itself: DILATED spans x 203–291, HYPERTROPHIC 408–569, RESTRICTIVE 659–795, putting the dividers at x ≈ 370 and x ≈ 606. A callout in the wrong slab does not merely mislabel a structure — it teaches the wrong phenotype.

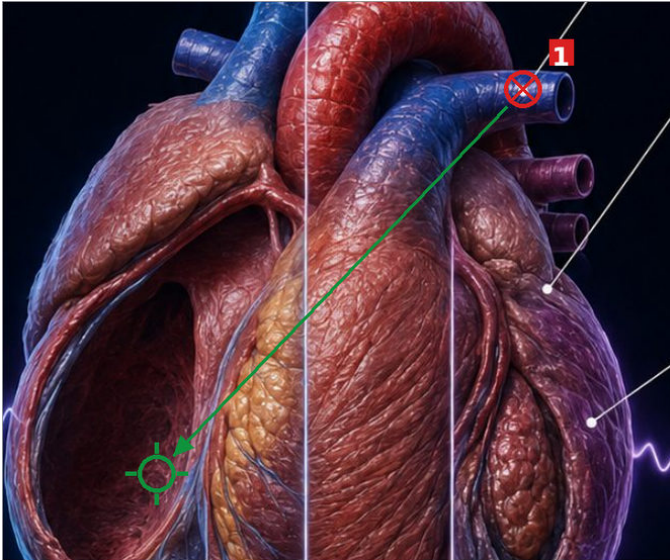


WHAT TO CHANGE

- 1 DILATED**
NOW (673,520) the RESTRICTIVE slab — and on a great vein, not a ventricular wall
MOVE TO (330,880) the thin wall of the dilated chamber, inside the left slab
- 2 HYPERTROPHIC**
NOW (696,707) the RESTRICTIVE slab — on an atrial-appendage bulge
MOVE TO (530,830) the solid thickened myocardium in the middle slab
- 3 RESTRICTIVE**
NOW (738,831) the RESTRICTIVE slab — CORRECT
MOVE TO stays
- 4 ARRHYTHMOGENIC**
NOW (706,1047) the RESTRICTIVE slab — and NO ARRHYTHMOGENIC SLAB IS DRAWN AT ALL
MOVE TO cannot be fixed by moving a leader: add a fourth slab, or drop the callout and carry ACM on its own panel

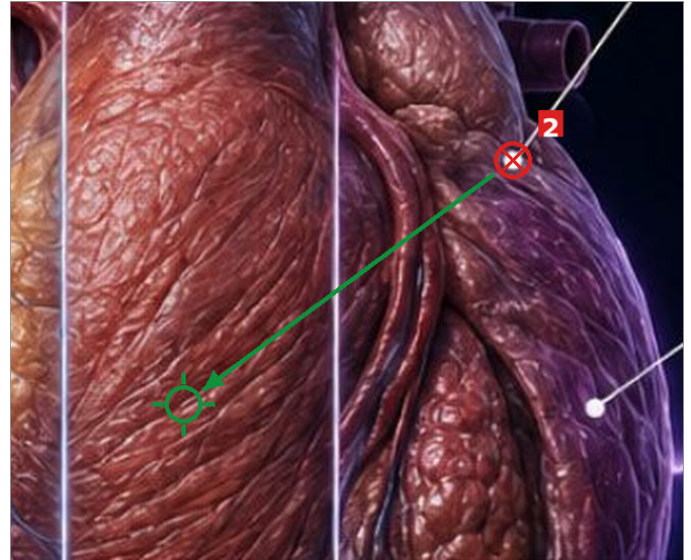
Cardiomyopathy — page 1 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



1 DILATED

(673,520) the RESTRICTIVE slab — and on a great vein, not a ventricular wall
 -> (330,880) the thin wall of the dilated chamber, inside the left slab



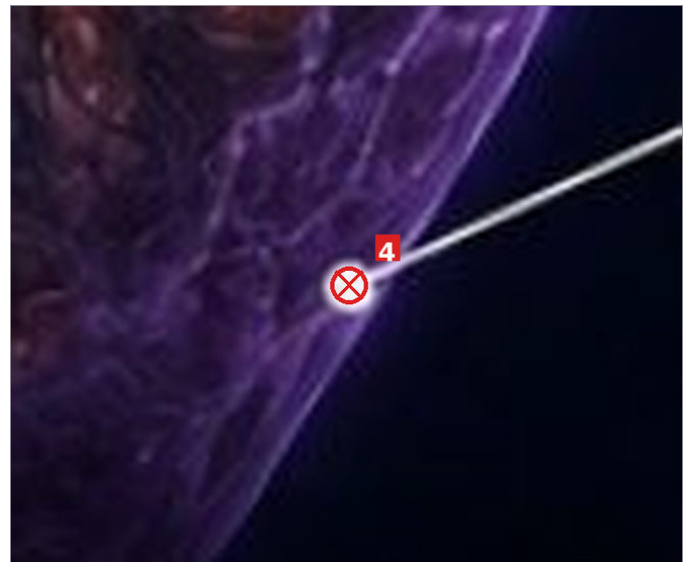
2 HYPERTROPHIC

(696,707) the RESTRICTIVE slab — on an atrial-appendage bulge
 -> (530,830) the solid thickened myocardium in the middle slab



3 RESTRICTIVE

(738,831) the RESTRICTIVE slab — CORRECT
 -> stays



4 ARRHYTHMOGENIC

(706,1047) the RESTRICTIVE slab — and NO ARRHYTHMOGENIC SLAB IS DRAWN AT ALL
 -> cannot be fixed by moving a leader: add a fourth slab, or drop the callout and carry ACM on its own panel

Cardiomyopathy — page 1 — notes

NOTES

As shipped, three of the four callouts point into the restrictive third of the figure. Re-ordering the label column is not required — with the endpoints above the four leaders fan without crossing (checked at every intersection). ACM is classically RIGHT-ventricular while the current leader points at the LV free wall, so a fourth slab would need to show the RV. A TRAP WORTH PASSING ON: there is a strong pale vertical line at $x = 469$ that reads like a divider and is not one — it sits inside the HYPERTROPHIC caption's own span, so it cannot be a slab edge.

Pericarditis — page 4

ECG strip · pericarditis-04.jpg · 1280×1920 px · 3 findings

Needs the strip REDRAWN, not re-arrowed.

Two of the five arrows name a finding the tracing does not contain, so there is no endpoint to move them to.



IMAGE 4 OF 10

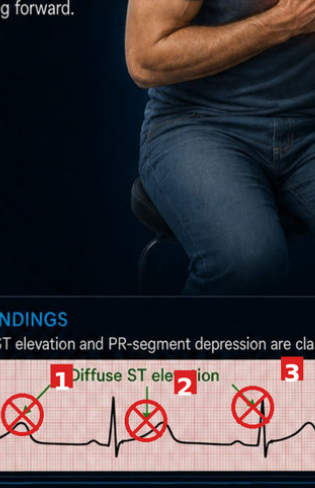
CARDIOLOGY

ACUTE PERICARDITIS

CLINICAL FEATURES

HOW ACUTE PERICARDITIS PRESENTS

Sharp, pleuritic chest pain is the hallmark symptom. It is typically worse with inspiration or lying flat and improves when sitting up and leaning forward.



POSITIONAL PAIN

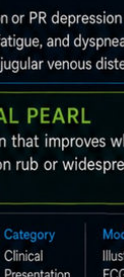


Better:
sitting forward



Worse:
lying flat

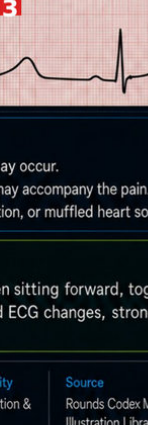
PERICARDIAL FRICTION RUB



Scratchy, grating sound best heard at the left lower sternal border.

ECG FINDINGS

Diffuse ST elevation and PR-segment depression are classic in acute pericarditis.



AT A GLANCE

- Diffuse ST elevation or PR depression may occur.
- Low-grade fever, fatigue, and dyspnea may accompany the pain.
- New hypotension, jugular venous distention, or muffled heart sounds are red flags.

EDUCATIONAL PEARL

Pleuritic chest pain that improves when sitting forward, together with a pericardial friction rub or widespread ECG changes, strongly suggests acute pericarditis.

Image Title	Category	Modality	Source	Evidence Level	RC
Clinical Features	Clinical Presentation	Illustration & ECG	Rounds Codex Medical Illustration Library	High	Rounds Codex Verified

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WHAT TO CHANGE

- 1 green arrow 1**
NOW (216,1222) ~73 px after R — the T-wave / ST-T peak; points PAST the ST segment
MOVE TO the ST segment — but see the note
- 2 green arrow 2**
NOW (410,1231) ~44 px after R — on the ST segment
MOVE TO acceptable as drawn
- 3 green arrow 3**
NOW (578,1210) 12 px BEFORE the R peak — on the QRS upstroke
MOVE TO the ST segment

Pericarditis — page 4 — close-ups

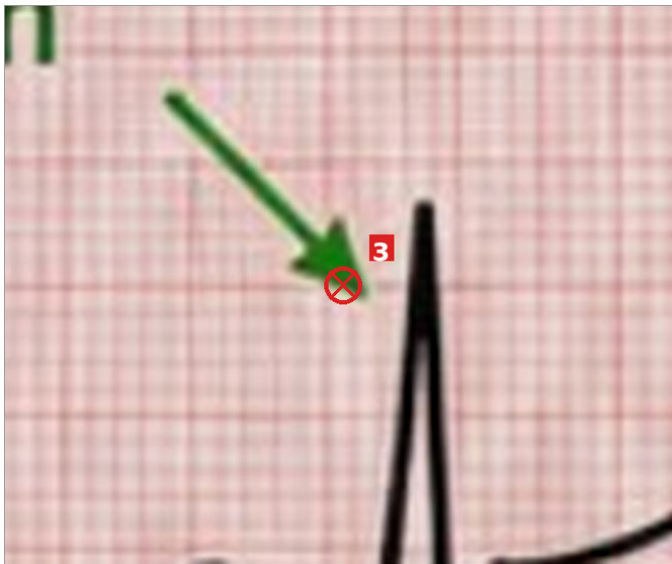
red crossed circle = where it ends now green plain circle = where it belongs



1 green arrow 1
(216,1222) ~73 px after R — the T-wave / ST-T peak;
points PAST the ST segment
-> the ST segment — but see the note



2 green arrow 2
(410,1231) ~44 px after R — on the ST segment
-> acceptable as drawn



3 green arrow 3
(578,1210) 12 px BEFORE the R peak — on the QRS upstroke
-> the ST segment

NOTES

The deeper problem is that the tracing does not contain the finding the arrows name — PR depression is not present in the drawn complexes. Moving the arrows cannot create it. The strip has to be redrawn with the abnormality actually present.